

Entero Healthcare Solutions Ltd

Buying India's Pharma Distribution One District at a Time



SCIENTIFIC INVESTING AND MAYANK AGARWAL

AUG 15, 2026



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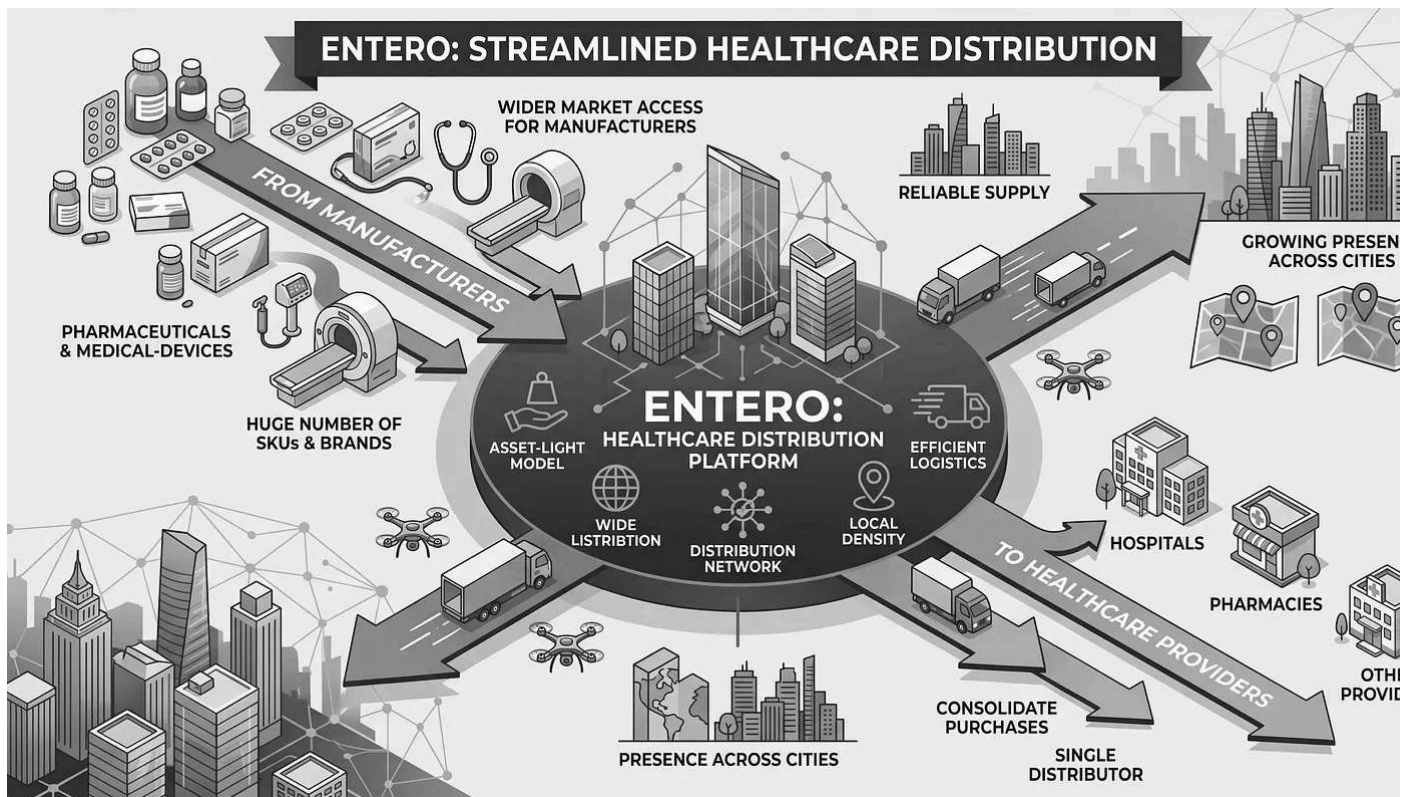


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Look at how pharmaceutical distribution is organised in any large economy, and a single number tells the entire story: **the share of the top three players.**

Start bigger for a second. According to Mordor Intelligence, the Indian pharmaceutical market stood at Rs. 4,97,000 crore (US\$ 57.61 billion) in 2025, is estimated at Rs. 5,20,000 crore (US\$ 60.32 billion) in 2026 and is expected to grow to 6,89,000 crore (US\$ 79.74 billion) by 2031, at a CAGR of 5.74%.

But there is a very basic and often overlooked part of the story: **how does the medicine actually get from the manufacturer to the patient?**

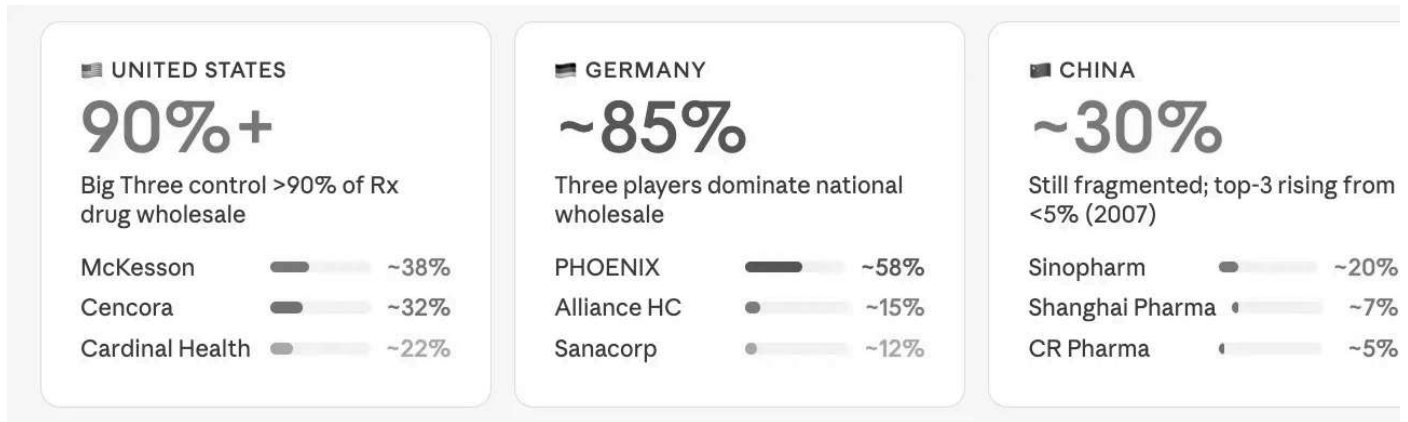
A drug leaving the factory does not magically appear on a pharmacy shelf. It has to move through a complex network of distributors, stockists, hospitals, pharmacies, and other healthcare providers before it reaches the end patient.

This may sound like a simple part of the healthcare value chain, but it is incredibly important. The distribution layer is the **plumbing of the pharmaceutical industry** largely invisible when it works well, but critical to ensuring that a life-saving drug reaches a chemist in Nashik as reliably as it does one in South Delhi.

This is what we will explore in this blog : the healthcare distribution value chain, how it is structured, why scale and consolidation matter, and where India stands compared with other large pharmaceutical markets. We will then bring this back to the Indian listed space and look at one company that is building a scaled platform in this critical but largely overlooked part of the healthcare ecosystem: **Entero Healthcare**.

In the United States, the top three : McKesson, Cardinal Health, AmerisourceBerg control roughly 90–95% of the market. **In Germany**, the top five hold 95–97%. **In China**, the top four sit at 40–45%. However, in India, top 3 players command less than 10% of market. Will it change? Can it change? Let us explore?

The themes across all markets



Theme 1 — the same forces, different speeds.

All three markets were consolidated by:

- (a) M&A that absorbed regional players,
- (b) Scale economics that made survival impossible below a threshold size, and
- (c) Sownstream customer concentration (pharmacy chains, hospital systems) that demanded nationwide supply reliability

Executive Summary

The U.S. pharmaceutical wholesale distribution industry is an extremely concentrated oligopoly dominated by three firms: McKesson Corporation, AmerisourceBergen (now Cencora, Inc.), and Cardinal Health. These companies collectively control well over 90% of the market by revenue (1) (2). In 2018-2019, their combined U.S. drug distribution revenues approached half a trillion dollars (with McKesson, AmerisourceBergen, and Cardinal all ranking in the top 15 of the Fortune 500 by revenue) (3) (4). This dominance has grown through decades of consolidation: mergers and acquisitions (5) (6). AmerisourceBergen's purchases of the H.D. Smith wholesaler and Walgreens' Alliance Healthcare and joint ventures with large pharmacy chains have steadily eroded smaller competitors (7) (8). At the same time, shifts in the drug market (such as generic price deflation and a surge in high-cost specialty drugs) have shaped distributor profitability (9) (10).

Source: Intuition Labs

These forces may look different across countries, but the underlying playbook is remarkably similar: **build scale, expand geographic reach, absorb smaller players, use that scale to become an increasingly important link between manufacturers and healthcare providers.**

The result is that pharmaceutical distribution tends to become a **scale game** over time. The companies that can spread their fixed costs across a larger volume of medicine

maintain a wider distribution network, and offer reliable service to increasingly large customers are better positioned to consolidate the market further.

To see how this plays out in practice, it is useful to look at two very different markets — **Cencora in the US and Sinopharm in China**. Their journeys offer a useful reference point for understanding what consolidation can look like when a fragmented distribution market begins to scale.

Cencora: M&A Over the years

Cencora's capital allocation is acquisition-led, with M&A accounting for the majority of total capital deployment. Over 2015–2025, the company deployed nearly ~\$19B toward acquisitions, highlighting M&A as a core growth lever rather than a supplemental strategy which is ~52% of total capital deployment

Capital Deployment

(in thousands)	Fiscal Years Ended September 30,				
	2019	2018	2017	2016	2015
Capex	\$310,222	\$336,411	\$466,397	\$464,616	\$231,585
M&A ^{4,5}	\$63,951	\$785,299	\$72,995	\$2,750,390	\$2,633,412
Dividends	\$338,974	\$333,041	\$320,270	\$288,477	\$253,919
Share Repurchases ^{6,7}	\$674,031	\$639,235	\$329,929	\$2,266,344	\$1,859,106

Capital deployment

Fiscal years ended

September 30 ¹	2025	2024	2023	2022	2021
Capex	\$667,981	\$487,173	\$458,359	\$496,318	\$438,217
M&A	\$4,291,872	\$100,201	\$2,153,110	\$152,305	\$5,725,660
Share repurchases	\$435,471	\$1,491,367	\$1,180,728	\$483,704	\$82,150
Dividends	\$437,081	\$416,168	\$398,752	\$391,687	\$366,648
Total	\$5,832,405	\$2,494,909	\$4,190,949	\$1,524,014	\$6,612,675

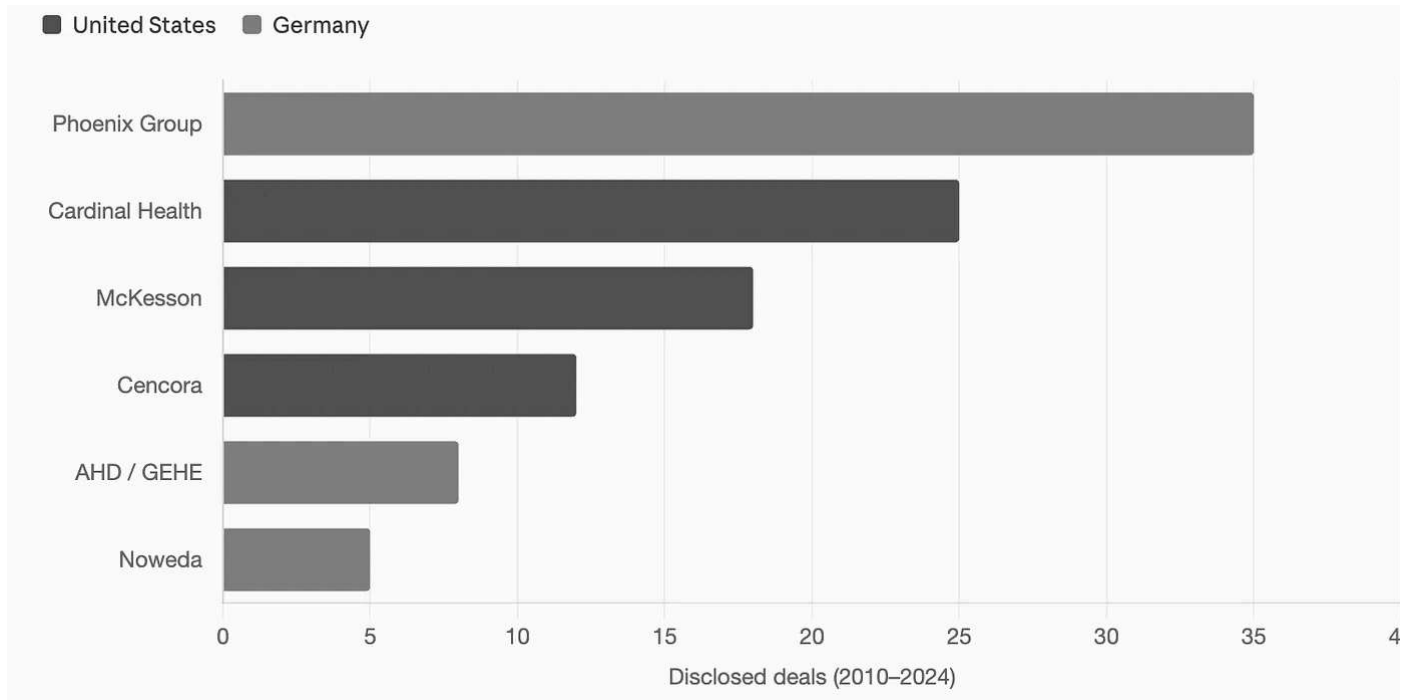
Sinopharm (China)

Sinopharm has built a much broader healthcare platform spanning pharmaceutical, medical equipment, retail chains and healthcare services. Its scale with 1,600 subsidiaries and around 180,000 employees shows how distribution can evolve into much larger healthcare ecosystem over time.



If we look at Overall picture of M&A over the years. The M&A activity across the 1 and Germany tells a similar story. Large players such as Cardinal Health, McKesson, Cencora and Phoenix Group have used acquisitions to expand their geographic footprint, customer relationships and scale.

The common thread is clear: consolidation is not just about getting bigger; it is about building a distribution network that becomes increasingly difficult for smaller players to replicate.



This question was asked to management in one of the concall if India would go same way how USA has been in terms of consolidation. As per management, India is moving and will move towards consolidation. However, USA is an extreme market from consolidation perspective. India may not achieve that kind of consolidation. However, between 10% for India and 90% for USA, there is a huge range, and this 10% consolidation should further go up

Ankur Gulati:

Hi. Just if you can give us some, I mean, how the industry has played out in the US and how do you see it playing out in the industry? Any lessons for us to learn from the US consolidation?

Prabhat Agrawal:

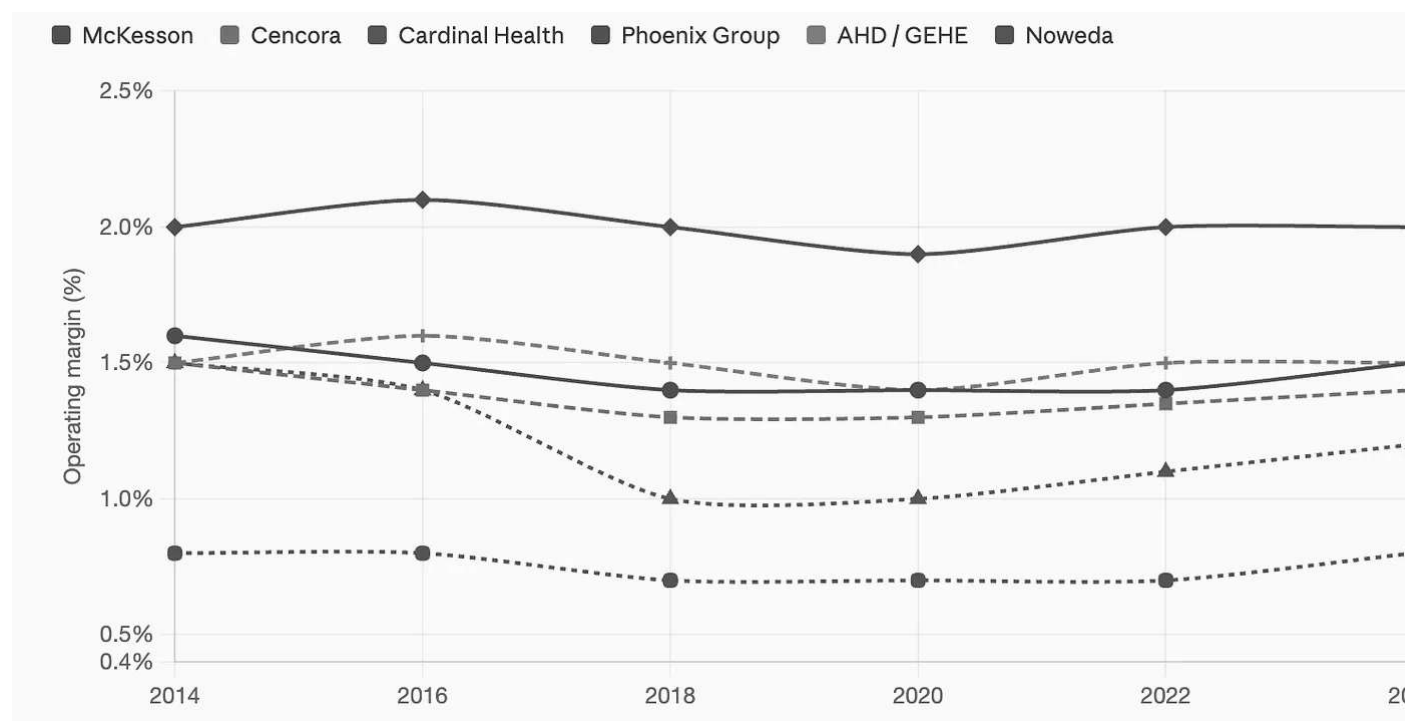
US is on the other extreme of consolidation. There are three distributors who are more than 90% market share. India is also moving towards consolidation, but it's not likely to go at that extreme level in near term at least.

Theme 2 — thin margins make scale the only weapon.

Metric	Pharma Industry Average	AmerisourceBergen (Estimated)	Cardinal Health (Estimated)	McKesson (Estimated)
Days Inventory Outstanding (DIO)	40-50 days	~45-50 days	~40-45 days	~45 days
Days Sales Outstanding (DSO)	40-50 days	~40-45 days	~40 days	~40 days
Days Payables Outstanding (DPO)	20-30 days	~22-25 days	~30-35 days	~25-30 days
Cash Conversion Cycle (CCC)	60-70 days	~60-70 days	~50-55 days	~55-60 days

Drug wholesale operates on extremely thin margins (~1–3%), making scale the primary competitive advantage. As a result, success depends on efficiently managing working capital particularly inventory turns and receivable collections while leveraging large transaction volumes to spread logistics, technology, and infrastructure costs.

Across major distributors, cash conversion cycles remain relatively high (~50–70 days), reinforcing the importance of disciplined inventory and payables management.



This highlights why strong operating cash flow generation is critical in pharmaceutical distribution. Given the industry operates on extremely thin margin any weakness in cash generation forces companies to rely more heavily on short-term borrowing to fund working capital needs such as inventory and receivables.

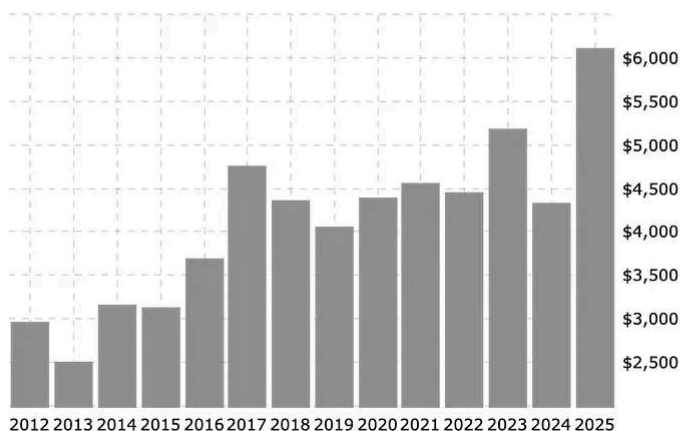
Higher borrowing increases interest expense, which can materially pressure already low operating margins and reduce net profitability. As a result, efficient cash conversion and disciplined working capital management become essential to sustaining profitability and funding growth without excessive leverage.

The operating cash flow (OCF) and operating profit trends across McKesson, Cardinal Health, and Cencora highlight an important point: **in pharmaceutical distribution, profitability alone is not enough the ability to convert that profit into cash is equally critical.**

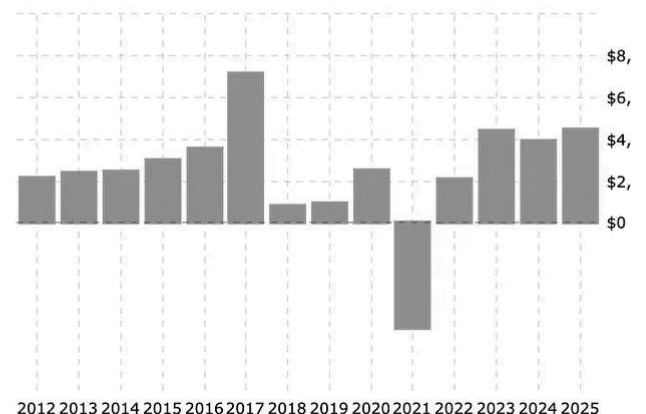
But what does this look like from the inside? **How do leading distributors manage working capital, cash conversion and leverage while operating on relatively thin margins?**

The following excerpts provide a glimpse of few companies and their respective CF chart over the years

MCK - Cash Flow from Operating Activities
Annual Values (Millions of US \$)

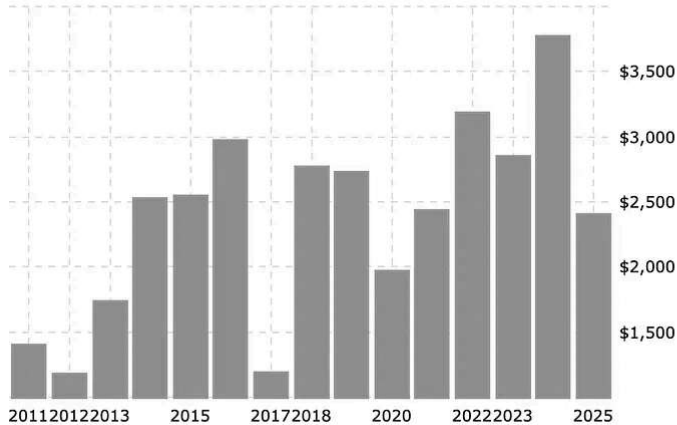


MCK - Operating Income
Annual Values (Millions of US \$)

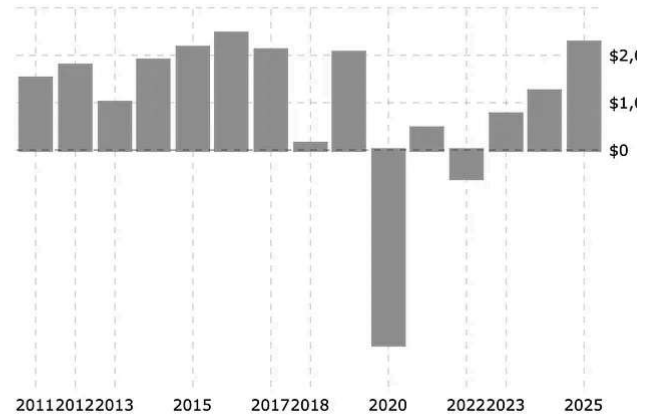


CAH - Cash Flow from Operating Activities

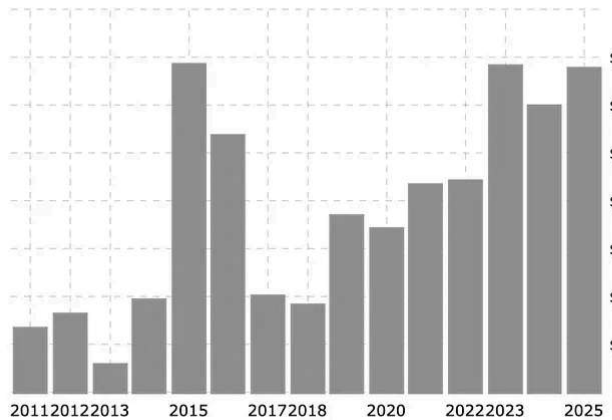
Annual Values (Millions of US \$)

**CAH - Operating Income**

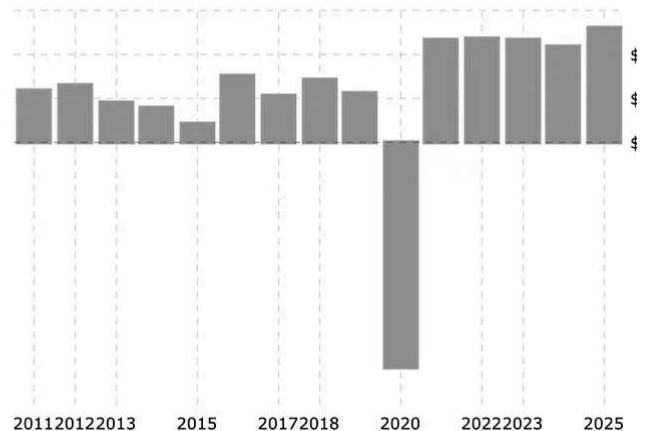
Annual Values (Millions of US \$)

**COR - Cash Flow from Operating Activities**

Annual Values (Millions of US \$)

**COR - Operating Income**

Annual Values (Millions of US \$)



The numbers reinforce this point. Across McKesson, Cardinal Health and Cencora, operating cash flow has generally remained strong and has improved meaningfully over the long term, even though operating profits have seen periods of volatility. The ability to consistently generate cash through the cycle is therefore a key differentiator in this business.

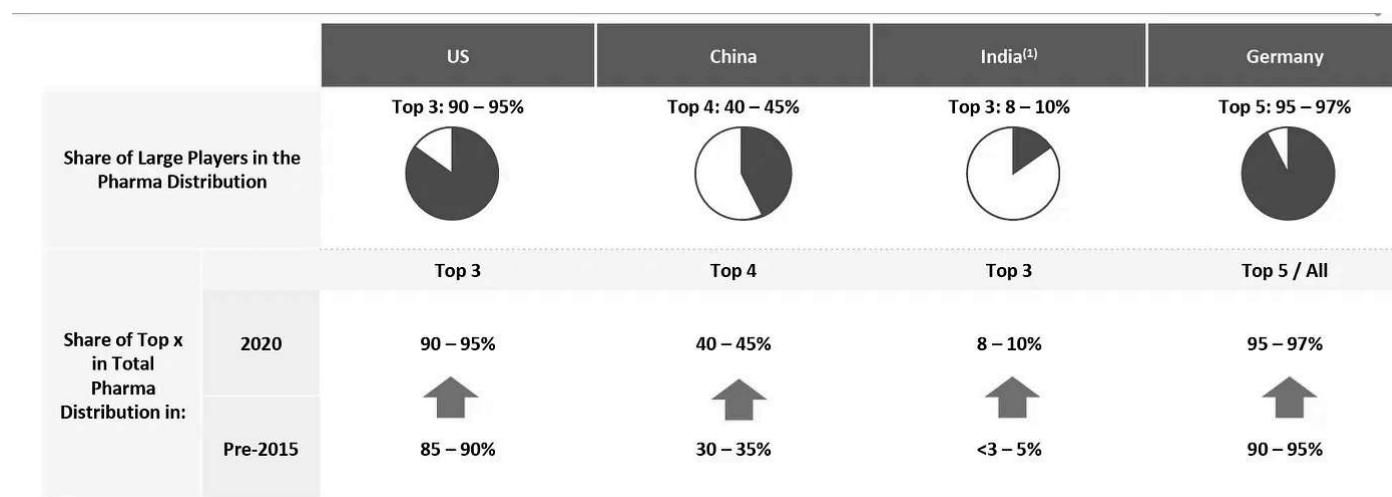
For a distributor operating on thin margins and handling large volumes, even a small improvement in working capital efficiency can have a meaningful impact on free cash flow. This makes cash conversion, inventory turns and receivable management just

important to track as revenue and EBITDA when evaluating a pharmaceutical distributor.

The global examples point to a common pattern: as **pharmaceutical distribution scales, consolidation tends to follow**. M&A, scale economics and increasing customer concentration and more product base and gradually push the industry towards a smaller number of large, integrated players.

But India has remained the structural anomaly.

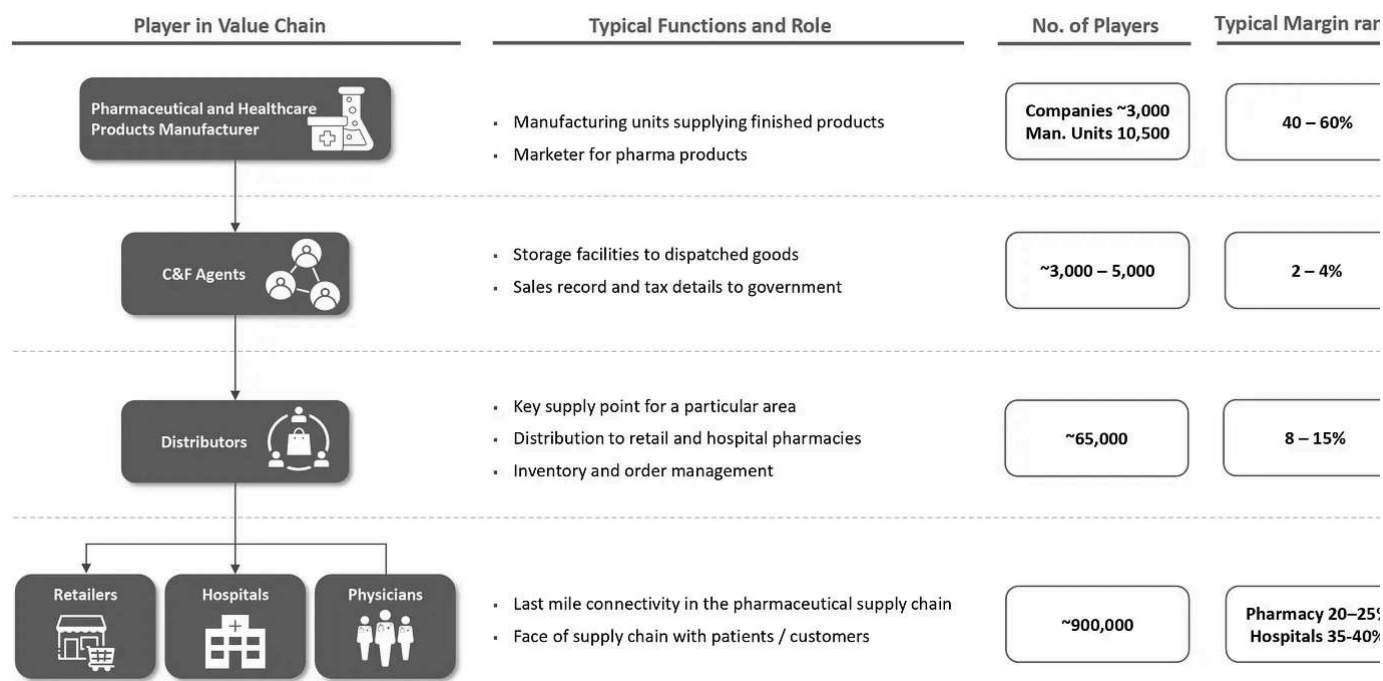
Despite being one of the world's largest pharmaceutical markets, **the top three pharmaceutical distributors together account for just 8–10% of the market**. The contrast with the US, Germany and even China is striking.



Why has India remained so fragmented?

As this is not a small gap. It is, perhaps, the single largest unconsolidated pool of distribution revenue in any major industry, anywhere. Roughly ₹2.7 trillion (≈USD \$ billion) of healthcare products move through India's distribution system each year, 70-75% of that volume is handled by traditional, regional, often single-state distributors who serve about 9,00,000 retail pharmacies.

India's pharmaceutical distribution market remains highly fragmented, with nearly ~65,000 distributors and ~900,000 retail touchpoints operating between manufacturer and end customers.



Unlike the U.S., where scale is concentrated among a few national players, India's supply chain still relies heavily on regional distributors and C&F agents for warehousing, inventory management, and last-mile delivery.

The value pool across the chain is uneven manufacturers capture the highest margin (~40–60%), while distributors operate on relatively thin margins (~8–15%), making working capital efficiency and scale critical for profitability. Retail pharmacies and hospitals continue to hold stronger economics due to their direct customer interface and higher service mix.

The industry is currently gradually shifting from fragmented regional networks to organized, technology-enabled distribution models. Rising demand for specialty drugs, e-pharmacy penetration, GST-led supply chain formalization, and increasing

compliance requirements are accelerating consolidation and creating opportunities for scaled distributors with strong logistics and cash flow capabilities.

The cold-chain requirements of biologics and the traceability rules for serialisation made small, paper-based distributors increasingly uneconomic. **As, the share of large distributors has gone from below 5% pre-2015 to 8–10% today**

Either consolidation in India's pharmaceutical distribution market is finally beginning or the long-held assumption of a fragmented ecosystem still holds true, making India an exception to global distribution trends. The thesis is inherently binary.

As scale, compliance, technology adoption, and working capital efficiency become increasingly important, the industry may gradually shift toward larger organized players with stronger supply chain capabilities. If that transition plays out, the implications for revenue growth, margin expansion, and return profiles could be significant.

This sets the stage to examine a company operating in this industry and how its revenues, margins, cash flows, and returns evolve as the market potentially moves from fragmentation toward consolidation.

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Entero Healthcare Solutions

Founded in January 2018 by Prabhat Agrawal (ex-CEO of Alkem Labs) and Prem Sen (ex- Director – Offering Development and Product Management with IQVIA), Entero is a healthcare-focused distribution roll-up. It went public in February 2024, raising ₹1,000 Cr from the issue.

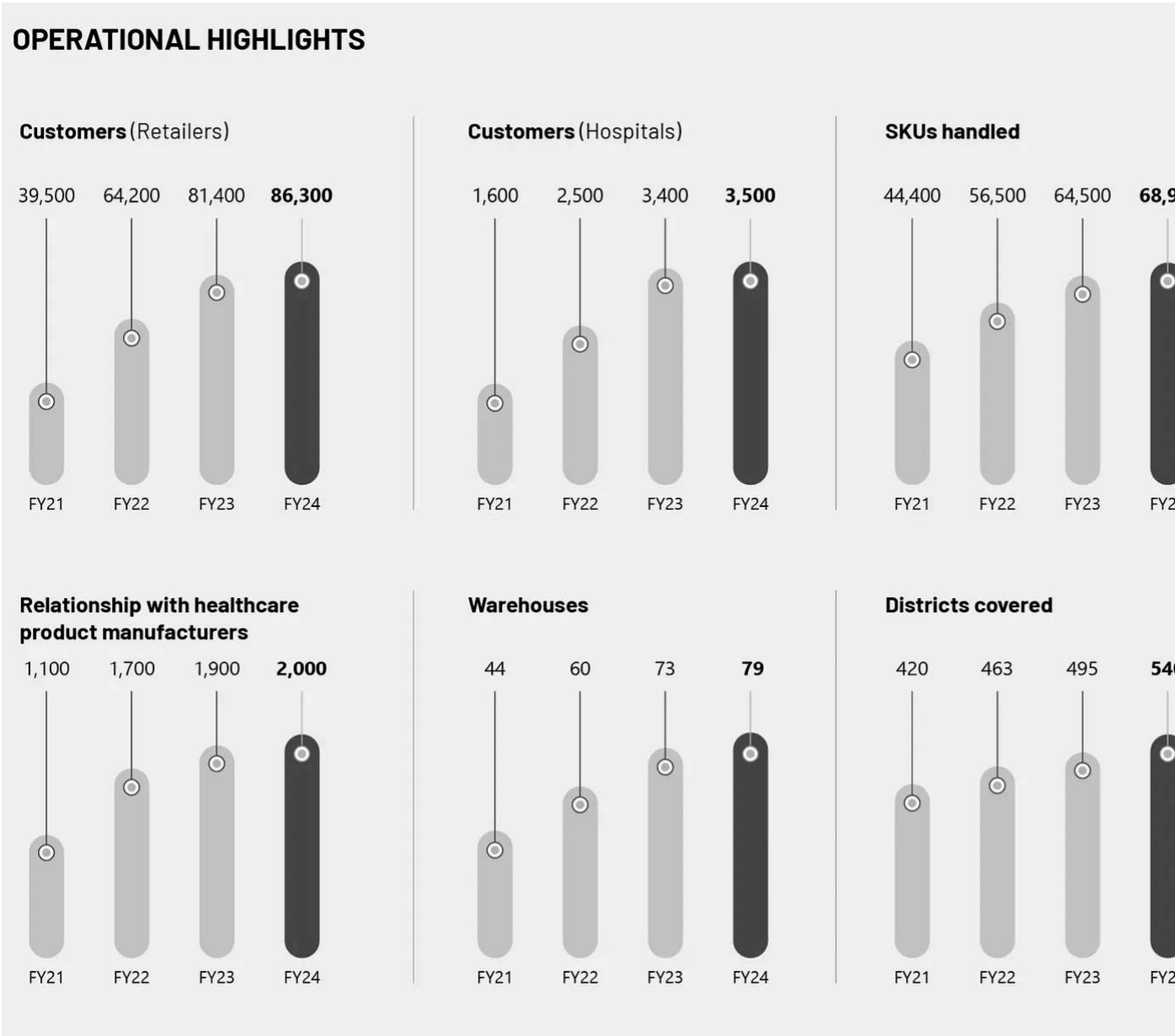
Entero's growth strategy reflects the early stages of consolidation within India's fragmented pharmaceutical distribution market. The company has acquired **34 healthcare distribution entities** over the years Pre-IPO, using M&A as a key lever to expand its geographic footprint, supplier relationships, and customer network.

The objects for which the Net Proceeds from the Offer shall be utilized are as follows:

Particulars	Estimated Amount (in ₹ million)
Repayment/prepayment, in full or part, of certain borrowings availed of by our Company	1,425.0
Funding of long-term working capital requirements of the Company and its Subsidiaries during Fiscals 2025 and 2026	4,800.0
General corporate purposes*	917.0
Pursuing inorganic growth initiatives through acquisitions*	2,370.0
Net Proceeds	9,512.0

This acquisition-led expansion has materially increased operating scale between FY23 and FY24:

- Retailer base expanded from **39,500** to **86,300**
- Hospital customers increased from **1,600** to **3,500**
- SKUs handled grew from **44,400** to **68,900**
- Warehouses scaled from **44** to **79**
- District coverage expanded from **420** to **540**



The strategy appears focused on integrating regional distributors into a larger organized network, allowing Entero to deepen market penetration while improving supply chain density and procurement reach.

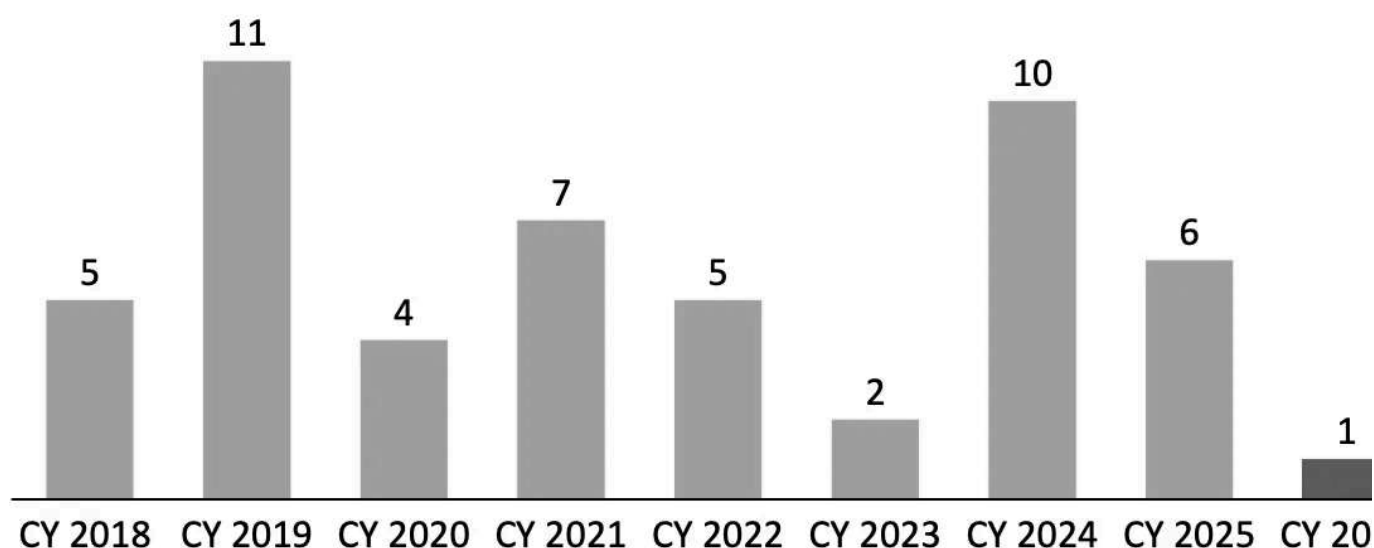
Also, Entero plans to maintain its strict criteria for acquisitions, targeting single-digit EV/EBITDA multiples.

Particulars	For the six months ended September 30,		For the Financial Year		
	2023	2022	2023	2022	2021
Number of entities acquired	1	3	3	8	
Amount invested in the acquisition of entities (₹ in millions)	226.97	245.74	245.74	1,469.24	21.0
Revenue recognized from entities (other than entities acquired during the financial year) (₹ in millions)	20,358.89	16,247.55	33,639.94	25,054.72	18,819.8
Revenue recognized from entities acquired during the financial year (₹ in millions)	41.63	248.72	836.23	1,416.27	212.2
Intra-group transactions* (₹ in millions)	(1,445.51)	(656.59)	(1,474.11)	(1,250.33)	(1,234.78)
Revenue from operations (₹ in millions)	18,955.01	15,839.68	33,002.07	25,220.65	17,797.3

*Intra-group transactions include eliminations of intra-group transactions and pre-acquisition revenues.

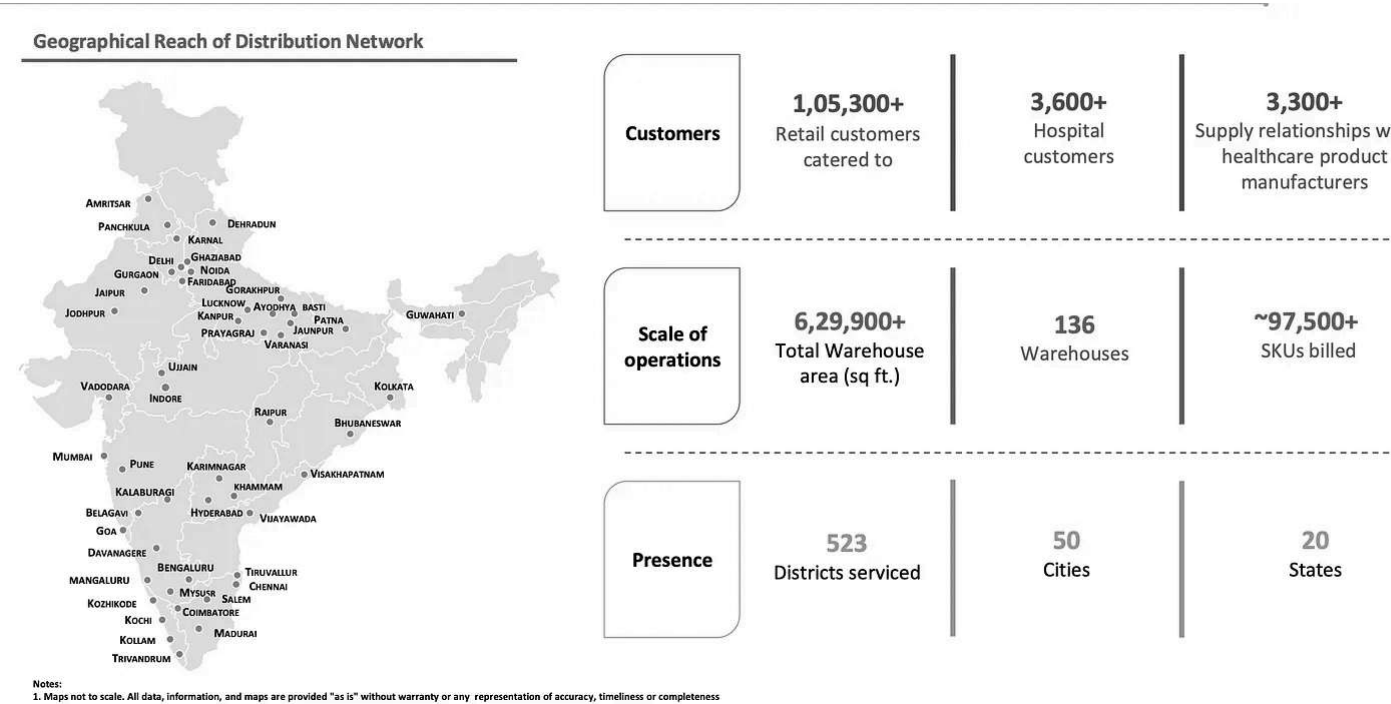
We are focussed on strategic acquisitions to consolidate our distribution presence across key markets and fuel our long-term growth. We have acquired 34 entities in the healthcare products distribution industry. Due to our acquisition track record and pan India approach, we have been able to consistently attract distributors to integrate with us. Moving forward, we plan to complement our organic growth with strategic acquisitions to enhance our customer and supplier network. We are actively pursuing expansion opportunities through strategic acquisitions of regional and local distributors.

51 Acquisitions Since Inception



Fast forward to FY26, it had completed 51 acquisitions since inception, served 1,05,300+ retailers (39,500 in FY21) , 3,600+ hospitals (1,600 in FY21), 136 warehouses

44 in FY21), ~97.5K+ SKUs (44.4K in FY21) and INR 6,591Cr of FY26 revenue a 30% CAGR over five years.



These numbers raise a simple question: can a pharma distributor grow through acquisitions while also generating healthy and sustainable returns?

To answer this, we need to look at Entero’s business model, unit economics, industry structure and financial performance.

Chapter 1: The Business at First Glance and Underneath

At first glance, Entero is a pharmaceutical wholesaler. It buys medicines, medical devices, surgical consumables, nutraceuticals, and over-the-counter products from manufacturers in bulk, stores them in 136+ leased warehouses spread across 50+ Indian cities, and supplies them to retail pharmacies and hospitals on a near-daily basis. About 90% of revenue is pharmaceuticals; the remaining 10% is the higher-

margin medical devices, in-vitro diagnostics, and surgical consumables a slice that set to grow materially given recent acquisitions.

That description is accurate, and it is also incomplete. It treats Entero as a logistics company, when the more useful frame is that Entero is selling two different products to two different customers, with a network effect connecting them.

What the company actually sells

At its core, Entero is selling **convenience and reliability**.

A pharmacy or hospital often has to deal with multiple distributors to source its full range of medicines one may specialise in chronic drugs, another in OTC products, another in surgical or cold-chain products. Each additional distributor means another relationship to manage, another bill to reconcile, and another ordering process.

Entero aims to simplify this by becoming a **one-stop supplier**. With 97K+ SKUs sourced from 3,600+ manufacturers, a single Entero account can meet a large part of a pharmacy's requirements.

Local density, not just pan-India scale

Pharma distribution is ultimately a local business. A pharmacy chooses a distributor based on availability, delivery speed, credit terms and reliability in its area. Similarly, a manufacturer launching a product in a new city needs a distributor with enough local reach to make that launch successful.

This means Entero cannot simply build one large national network and expect the benefits to flow everywhere. **It needs to build density city by city and cluster by cluster.**

Once that local density is established, Entero can use its central procurement, technology and distribution infrastructure to bring scale benefits across the network.

That is where the real operating leverage can begin to emerge.

Our relationships with such a huge range of healthcare product companies, nationwide pharmacies and hospital reach, robust and technology-enabled last mile distribution infrastructure creates a very strong business moat and differentiated value proposition for both our suppliers and customers.

This is why the acquisition strategy is pan-India in form but local in effect as the acquired entity brought **existing retailer relationships, local team relationships, and warehouse footprint**. Entero brought capital, technology, procurement leverage, and access to a wider product portfolio. Both ends benefit; both ends are needed.


V. Holding, Subsidiary and Associate Companies (including joint ventures)
23. (a) Names of holding / subsidiary / associate companies / joint ventures

S. No.	Name of the holding/ subsidiary/ associate companies/ joint ventures (A)	Indicate whether holding/ Subsidiary/ Associate/ Joint Venture	% of shares held by listed entity	Does the entity indicated at column A, participate in the Business Responsibility initiatives of the listed entity? (Yes/No)
1.	R S M Pharma Private Limited	Subsidiary	100.00	Yes
2.	Jaggi Enterprises Private Limited	Subsidiary	100.00	Yes
3.	G.S.Pharmaceutical Distributors Private Limited	Subsidiary	100.00	Yes
4.	Novacare Healthcare Solutions Private Limited	Subsidiary	100.00	Yes
5.	Chhabra Healthcare Solutions Private Limited	Subsidiary	100.00	Yes
6.	Galaxystar Pharma Distributors Private Limited	Subsidiary	100.00	Yes
7.	Sundarlal Pharma Distributors Private Limited	Subsidiary	100.00	Yes
8.	Millennium Medisolutions Private Limited	Subsidiary	100.00	Yes
9.	Getwell Medicare Solution Private Limited	Subsidiary	100.00	Yes
10.	Vasavi Medicare Solutions Private Limited	Subsidiary	100.00	Yes
11.	Avenues Pharma Distributors Private Limited	Subsidiary	100.00	Yes
12.	Chirag Medicare Solutions Private Limited	Subsidiary	100.00	Yes
13.	Chethana Healthcare Solutions Private Limited	Subsidiary	100.00	Yes
14.	Rada Medisolutions Private Limited	Subsidiary	100.00	Yes
15.	Chethana Pharma Private Limited	Subsidiary	100.00	Yes
16.	SVMED Solutions Private Limited	Subsidiary	100.00	Yes
17.	Sesha Balajee Medisolutions Private Limited	Subsidiary	100.00	Yes
18.	Barros Enterprises Private Limited	Subsidiary	100.00	Yes
19.	Calcutta Medisolutions Private Limited	Subsidiary	100.00	Yes
20.	Sree Venkateshwara Medisolutions Private Limited	Subsidiary	100.00	Yes
21.	Curever Pharma Private Limited	Subsidiary	100.00	Yes
22.	Atreja Healthcare Solutions Private Limited	Subsidiary	100.00	Yes
23.	City Pharma Distributors Private Limited	Subsidiary	100.00	Yes
24.	Western Healthcare Solutions Private Limited	Subsidiary	100.00	Yes
25.	Swami Medisolutions Private Limited	Subsidiary	100.00	Yes
26.	New RRPD Private Limited	Subsidiary	100.00	Yes
27.	New Siva Agencies Private Limited	Subsidiary	100.00	Yes
28.	Sri Rama Pharmaceutical Distributors Private Limited	Subsidiary	100.00	Yes
29.	Sri Parshva Pharma Distributors Private Limited	Subsidiary	100.00	Yes
30.	Saurashtra Medisolutions Private Limited	Subsidiary	100.00	Yes
31.	SVS Lifesciences Private Limited	Subsidiary	100.00	Yes
32.	S.S. Pharma Traders Private Limited	Subsidiary	100.00	Yes
33.	Entero R.S. Enterprises Private Limited	Subsidiary	100.00	Yes
34.	Dhanvanthri Super Speciality Private Limited	Subsidiary	80.00	Yes
35.	Avenir Lifecare Pharma Private Limited	Subsidiary	80.00	Yes
36.	Devi Pharma Wellness Private Limited	Subsidiary	100.00	Yes
37.	Gourav Medical Agencies Private Limited	Subsidiary	80.00	Yes
38.	Srinivasa Lifecare Private Limited	Subsidiary	70.00	Yes
39.	Peerless Biotech Private Limited	Subsidiary	76.00	Yes
40.	Sai Pharma Distributors Private Limited	Subsidiary	70.00	Yes
41.	Ujjain Maheshwari Pharma Distributors Private Limited	Subsidiary	100.00	Yes
42.	Ramson Medical Distributors Private Limited	Subsidiary	70.00	Yes
43.	Sai RK Pharma Private Limited	Subsidiary	70.00	Yes
44.	Well Wisher Pharma Private Limited	Subsidiary	70.00	Yes
45.	Anand Medilink Private Limited	Subsidiary	80.00	Yes
46.	Ace Cardiopathy Solutions Private Limited	Subsidiary	60.00	Yes
47.	Bioaide Technologies Private Limited	Subsidiary	80.00	Yes
48.	Anand Chemiceutics Private Limited	Subsidiary	51.51	Yes

Per-acquisition economics

The clearest way to understand Entero's M&A maths is the explanation Prabhat Agrawal gave on an early earnings call.

Take a regional distributor with ₹100 of annual revenue, generating ₹4 of EBITDA (4% margin, typical for a sub-scale distributor running on basic procurement and rented warehousing). To run a ₹100 revenue book, that distributor must employ roughly ₹15–16 of working capital (receivables less payables, plus inventory)

Now, coming to using inorganic to do that as I told you typically we buy these targets on an EV EBITDA multiple basis and EV EBITDA multiple basis could be mid to high EV EBITDA single digit, mid to high single digit, EV EBITDA multiple basis and if you look at economic of return.

Typically, let's I give you an example. For example, if you buy a distributor whose sales is, for example, INR100 and let's say he's making INR4 of EBITDA for example, or INR5 of EBITDA. Let's use, for example, INR4 of EBITDA. Even if we value him at 6 times or 7 times let's say 6 times then we will be like INR24 of enterprise value. To run a INR100 business he would have definitely employed INR15 to INR16 of working capital.

So we are paying INR24 to acquire a revenue of INR100 which will deliver us an EBITDA of INR4 and along with it we are also getting INR16 of working capital. So the goodwill, the intangible part that we are paying is around INR8 and if we -- even if we don't do much with the business and the business continues to deliver only INR4 we will get back our money in 2 years

Source: Q4 FY24 Concall

Entero buys at, typically, 6x EV/EBITDA i.e., ₹24 of enterprise value. Of that ₹24, the seller hands over ₹16 of working capital embedded in the business. The net cash

outflow is therefore around ₹8 for ₹100 of revenue and ₹4 of EBITDA recurring per year. The headline EV/EBITDA looks like 6x, but the cash-on-cash payback is 2 years.

Per-warehouse economics — the asset-light truth

Entero is often described as a “logistics” or “supply chain” company, which can make the business appear capital-intensive. **In reality, the model is relatively asset-light.**

The warehouses are largely leased, while the capex required inside them is limited to racks, cooling systems and basic IT infrastructure. As management puts it, “**to do one warehouse, you would not need more than ₹30–40 lakh of capex.**”

This is important because Entero can add distribution capacity without having to make large investments in physical assets. **The key investment is therefore not in owning warehouses, but in building network density, inventory and customer relationships.**

Prabhat Agrawal:

So as you know, our business is not capital intensive. It's only working capital intensive. From a fixed assets point of view, we have a very small, fixed asset base because most of the -- not most, all of the warehouses are on leases. So, the only equipment that we would put there are storage equipments and cooling equipments to maintain the temperature and stuff like that.

So capex point of view, it's very minimal compared to the revenues that we generate from the warehouse. Probably to do one warehouse, you would not need more than INR30-40 lakhs of capex.

Source: Q3FY26 Concall

The trajectory of ROCE from below 2% in FY21 to nearly 10% in FY26 is the single most important number in the Entero story.

It reflects two compounding effects: the EBITDA margin has climbed (the next chapter unpacks why), and the asset base has scaled efficiently (no capex bloat). Both

effects need to continue for the ROCE expansion to extend; if either stalls, the trajectory flattens.

Ratios

Consolidated Figures in Rs. Crores / View Standalone

HEA

	Mar 2020	Mar 2021	Mar 2022	Mar 2023	Mar 2024	Mar 2025	Mar 2026
Debtor Days	63	50	54	57	57	59	
Inventory Days	57	54	49	41	43	52	
Days Payable	32	22	22	25	24	31	
Cash Conversion Cycle	88	82	81	73	77	80	
Working Capital Days	57	54	41	32	48	65	
ROCE %		1%	1%	5%	7%	9%	

The Operating Margin levers

Entero has crossed from 1.0% EBITDA margin in FY22 to 4.0% in FY26 and 5% as of Q1FY27. The margin expansion has not come from one source; it has come from five reinforcing levers.

Profit & Loss

Consolidated Figures in Rs. Crores / View Standalone

HEATMAP

RELATED PARTY

PROJECT

	Mar 2020	Mar 2021	Mar 2022	Mar 2023	Mar 2024	Mar 2025	Mar 2026
Sales +	1,350	1,773	2,522	3,300	3,922	5,096	6,591
Expenses +	1,326	1,758	2,498	3,236	3,810	4,924	6,325
Operating Profit	23	15	24	64	112	172	266
OPM %	1.7%	0.8%	1.0%	1.9%	2.9%	3.4%	4.0%

Understanding the levers separately matters because their durability differs.

1. The Margin Benefit of Scale and Commercialisation. Gross margin improvement (80bps in FY26) comes from two broad levers: **better purchasing economics from scale** and **higher margins from taking on a larger commercial role**. The first improves with scale, while the second depends on deeper manufacturer relationships and greater value addition.

So why we are able to get a better margin just because of our scale. Once you buy so much those quantities, of course, you can enjoy economics of purchasing, right. So that's number one. Secondly, in many places, as we said, we are playing a commercial role. Commercial role means we are also responsible for sales and demand generation.

In that scenario, the gross margin that the companies give us is much higher than the gross margin that they would give for a pure fulfillment role. So the impact of -- the reason for increase in gross margins is coming from basically these 2 reasons. One is the scale-led purchasing economies and secondly is more commercial role being played.

When I say commercial role, when I say for many companies, we are exclusive distributor for the entire India market, right. So we are doing end-to-end work for them from developing the customer, service, installation of equipment, creating the distributor network in the country, launching new products. So a full agency model as we say in other countries where we have full commercial role we are playing. Once you play a full commercial role, you would certainly expect much better gross margins than when you would play just a demand fulfiller role.

Source: Q4 FY26 Concall

2. Product mix toward higher-margin categories. Entero's recent acquisitions are helping it expand beyond traditional pharmaceutical distribution into **MedTech and diagnostics**, while also adding new products, customers and manufacturer relationships. The acquired businesses strengthen its presence across areas such as cardiology, IVD, lab consumables and medical devices.

Now coming to inorganic growth progress. We closed acquisitions of Anand Medilink in Pulmonology, Ace Cardiopathy, Bioaide Technologies and Anand Chemiceutics in MedTech segment. Each of these acquisitions further expands our distribution reach, widens our product segments and builds new capabilities for us. Particularly in MedTech segment, we have added scale in IV Catheters, Cardiology, POCT devices, lab consumables, etc.

Importantly, this is not just a revenue expansion opportunity. Management expects the integrated MedTech business to generate **₹1,000+ crore of annualised revenue**, with **70–90 bps improvement in gross margin** and **50–75 bps improvement in EBITDA margin** on a pro-forma basis

KEY COMPANIES ACQUIRED

Bioaide Technologies Pvt. Ltd.

The Company focused on high-growth segments including interventional cardiology, ENT, wound management and neuro psychiatry solutions. With strong relationships across hospitals and healthcare providers, the company enhances Entero's MedTech capabilities, expands its specialized product portfolio, and strengthens its presence in value-added healthcare markets.

BioAide
Technologies Pvt Ltd



Medtronic Resolute Onyx™
Zotarolimus-Eluting
Coronary Stent System

An advanced drug-eluting coronary stent featuring cobalt alloy platform and the proven zotarolimus-eluting polymer. Offers outstanding deliverability, sustained drug release, and proven long-term safety.



Sechrist Hyperbaric Oxygen Therapy Critical Care / Hyperbaric Medicine

An advanced hyperbaric oxygen therapy system delivering pressurized oxygen treatment to support wound healing, infection management, and recovery across critical care and specialized therapeutic applications.



Medtronic Quad CRT-P
Cardiac Rhythm Management / Cardiac Resynchronization Therapy

An advanced cardiac resynchronization therapy pacemaker designed to improve heart function, optimize rhythm coordination, and support enhanced quality of life for heart failure patients.



BrainsWay Deep TMS™
Neurostimulation / Mental Health Technology

An advanced non-invasive deep transcranial magnetic stimulation system designed to treat neurological and mental health disorders through targeted brain stimulation, enhancing therapeutic outcomes with precision-driven technology.



EBD Vessel Sealing
Advanced Surgical Systems / Electrosurgical Energy Platform

An advanced vessel sealing generator designed for precise tissue dissection and reliable vessel sealing, enhancing surgical efficiency, control, and safety across minimally invasive and open surgical procedures.



MED-EL Cochlear Implant
Hearing Solutions / Implantable Audiology Device

An advanced cochlear implant system designed to restore hearing through innovative audio processing and implant technology, enabling improved speech perception and enhanced quality of life for patients.

Ace Cardiopathy Solutions Pvt. Ltd.

A specialized MedTech distributor focused on the cardiovascular care segment. The company provides advanced cardiology devices and solutions to hospitals and healthcare providers through strong manufacturer partnerships and customer relationships. Its acquisition strengthens Entero's presence in high-growth MedTech categories and expands its healthcare solutions portfolio.



Balloon Catheter
Structural Heart / Balloon Valvuloplasty

A high-performance balloon catheter designed for precise cardiac valve dilation, enabling controlled expansion, improved blood flow, and enhanced procedural efficiency during minimally invasive valvuloplasty interventions.



CPAP System
Respiratory Care / Sleep Therapy

An intelligent CPAP therapy system designed to support sleep apnea management through automated airflow delivery, enhanced patient comfort, and connected monitoring for improved respiratory health outcomes.



VACS® II / VACS® III
Percutaneous Transluminal Valvuloplasty (PTV) Catheter

Percutaneous transluminal valvuloplasty (PTV) catheter for use in interventional adult and pediatric cardiology.



ASAHI Gaia Next
Asahi guidewire

A premium CTO guidewire engineered for precise lesion crossing in complex coronary interventions. It delivers outstanding torque response, controlled penetration, enhanced durability, and excellent steerability, enabling confident navigation through challenging occlusions.



Terumo Stents
(Ultimaster™ Tansel™ Sirolimus Eluting Coronary Stent System)

A next-generation biodegradable polymer, sirolimus-eluting coronary stent engineered for exceptional deliverability in complex coronary interventions. Built to support rapid vessel healing while delivering proven long-term safety and clinical performance.



VIVO ISAR Translumina Stent
Polymer-Free Sirolimus Eluting Coronary Stent System

A next-generation polymer-free sirolimus-eluting coronary stent featuring innovative dual-drug technology (sirolimus + Probucol). Built on a cobalt-chromium platform with a microporous surface and albumin coating. It delivers controlled drug release, promotes faster healing.

Anand Chemiceutics Pvt. Ltd.

STRENGTHENING OUR DIAGNOSTICS & MEDTECH PLATFORM

ANAND
CHEMICEUTICS

A leading distributor in the in-vitro diagnostics (IVD) and MedTech segment, with an established presence spanning both consumables and capital equipment across the diagnostics value chain. ACPL serves a diversified base of 1,500+ customers across pan-India, including corporate hospital chains, standalone hospitals, chain and standalone diagnostic laboratories, and a wide network of sub-distributors — reflecting deep market penetration across both metro and emerging healthcare markets. The acquisition strengthens Entero's presence in diagnostics, expands its MedTech footprint, and enhances access to a rapidly growing healthcare category.



APPLIED GENOMICS Our New Frontier in Precision Medicine

Anand Chemiceutics' Applied Genomics Division brings end-to-end genomics capabilities to the healthcare ecosystem - from sample extraction and quality control to next-generation sequencing and data analysis. With a focus on Oncology, Infectious Diseases and Rare Disorders, the division is building capabilities to accelerate the adoption of precision medicine in India.

Key Focus Areas



INFECTIOUS DISEASES



ONCOLOGY



RARE DISORDERS



Alinity
Clinical Chemistry, Immunoassay and Integrated Systems

Abbott Alinity is a family of automated laboratory analyzers designed for clinical chemistry, immunoassay, hematology, and molecular diagnostics.



Evolis and Evolis Elis

The Evolis is an automated ELISA processing system used for enzyme-linked immunosorbent assays in clinical laboratories.



DNA Sequencing System
Molecular

The world's first turnkey, automated NGS system. Nucleic acid to report in a single day with only two user touchpoints & superior sequencing performance based on Omicron readout data from GISAID



Genexpert®
Advanced Polymerase Chain Reaction System

The GeneXpert® System fully integrates and automates sample extraction, amplification, and detection — all in one cartridge. On-demand. Just load a biological sample and the system does the rest.



BS-240 Machine
Up to 200 T/H chemistry analyzers

Smart-sampling technology enables automatic hemolysis preparation for HbA1c test. Pretreatment-free operation guarantees high productivity.



SYS200
Fully Automatic, Discrete, Stat Priority Instrument

The SYS200 generally refers to a Sys automated hematology analyzer as complete blood count (CBC) and ac blood cell analysis.



BD BACTEC-960
Microbiology

The WHO endorsed the Use of Liquid Culture for the detection & Drug Susceptibility Testing (DST) of Mycobacteria. Gold standard Technology (Liquid Culture).



VARIANT II TURBO
HbA1c Testing

A fully automated system designed primarily for high-volume HbA1c testing to aid in the diagnosis and monitoring of diabetes.



Auto DELFIA
HbA1c, Thalassemia Testing

Auto DELFIA is a fully automatic & reproducible quantitation of anal commonly used in prenatal screen programs.



IH-500
Blood Bank

Fully Automated Walkaway Immuno-hematology Analyze. The innovative motor technology inside the arm provides a maintenance free transport component and increases the liability of the entire instrument



ABL9 Blood Gas Analyser
Blood Gas Analyser

A point-of-care instrument used to measure blood gases and electrolytes in critically ill patients. It is applied in Intensive Care Units (ICU) & Emergency departments.



Phadia 250
Allergy Testing

An automated immunoassay anal for allergy and autoimmune disease diagnostics by measuring specific antibodies and autoimmune mar

The larger opportunity is to use Entero's existing distribution network to scale the higher-value categories, while gradually improving the overall margin profile of the business.

As I have already guided, post-completion and integration of all these businesses, annualized revenue will cross over INR1,000 crores in MedTech segment. There will be a positive impact of 70 to 90 bps on gross margin and 50 to 75 bps on EBITDA margin on a pro forma basis after all the acquisitions are integrated with us. This segment represents an attractive growth opportunity for Entero along with margin enhancement potential. Overall, we are very well

Source: Q3 FY26

3. Operating leverage on a fixed cost base. Entero's employee cost was INR 245Cr in FY25 against ₹5,096 Cr revenue (~3.8% of revenue), down from 4.6% in FY22

4. Technology-Driven Operating Leverage: Entero's technology platform can further support margin expansion by improving the efficiency of its distribution network. Real-time inventory and order visibility, automated ordering, sales-force productivity tools, and its hub-and-spoke network can help the company handle higher volumes without a proportional increase in operating costs

Proprietary Technology Platform with Integrated Business Intelligence Tools and Solutions

Technology-Focused Approach Anchored on Our Proprietary Integrated Tech Platforms and Business Intelligence Tools...

Single-interface platform to pharmacies



...To Grow our Operations and Bring Efficiencies in the Healthcare Products Distribution Ecosystem



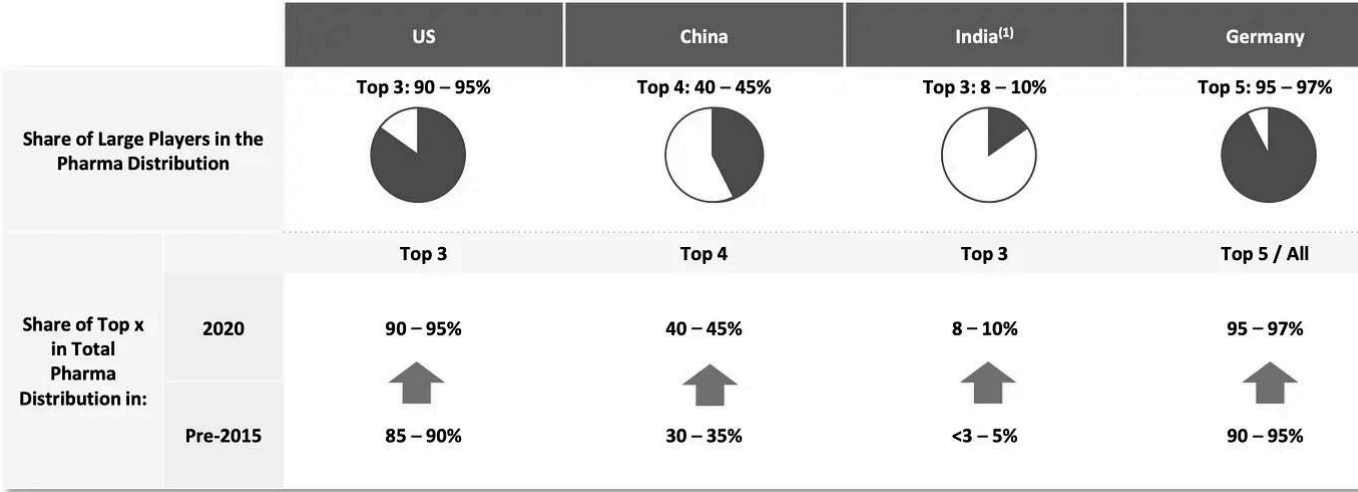
The platform also creates better data visibility for manufacturers, including secondary sales, inventory and micro-market demand, which can strengthen Entero's commercial role.

Overall, Entero's EBITDA margin expansion from 1.0% in FY22 to 4.0% in FY26 and 5.0% in Q1 FY27 reflects multiple factors working together: better procurement economics, higher-margin categories, commercial value-add and operating leverage. Importantly, these are not independent drivers; as the business scales, they can reinforce each other.

This also ties back to the global examples discussed earlier. In markets such as the US and Germany, margin expansion has been supported not only by working-capital efficiency, but also by acquiring higher-margin businesses, adding value-added services and building system-driven processes that improve operating efficiency. Entero appears to be pursuing a similar path, with acquisitions, higher-margin MedTech categories and increasing scale working alongside operational leverage.

Chapter 2: Industry Structure

We have looked at how pharmaceutical distribution works globally, why scale and consolidation matter, and why India remains significantly more fragmented. We have also seen how Entero is building scale through its distribution network, acquisition of higher-value categories and technology.



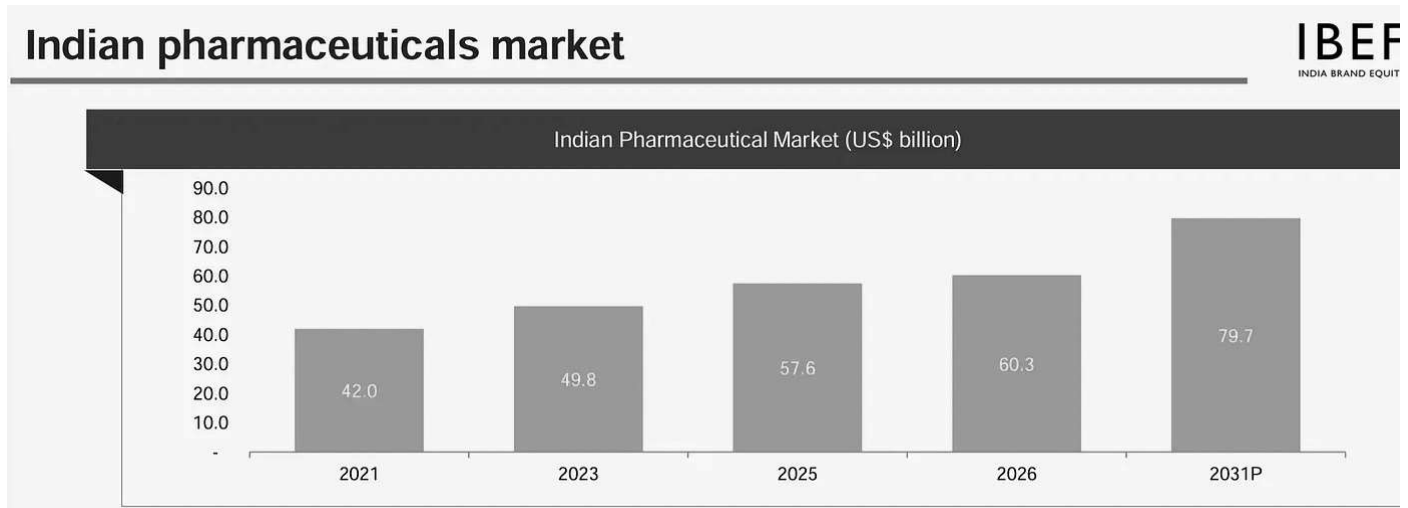
The next step is to put numbers around this opportunity.

How large is the addressable market for Entero? How much of this market can it realistically serve, and what could the business look like if it continues to gain share?

Large and Expanding Healthcare Distribution Opportunity

According to Mordor Intelligence, the Indian pharmaceutical market was valued at ₹5.20 lakh crore in 2026 and is expected to reach ₹6.89 lakh crore by 2031, implying 5.8% CAGR in INR terms.

The medical devices market adds another meaningful pool. It stood at ₹1.61 lakh crore in 2026 and is projected to reach ₹2.48 lakh crore by 2031, implying a much faster ~9.0% CAGR



The more interesting point for Entero is that its opportunity can grow through **two different avenues**: the underlying healthcare market itself is expanding, while the company can simultaneously increase its share of the distribution wallet.

Pharma provides the large existing base, while **MedTech and diagnostics provide a additional, faster-growing pool with potentially better margins**. Entero's recent acquisitions and expansion into categories such as IVD, cardiology, lab consumable and medical devices are therefore not just adjacent businesses they expand the company's addressable market.

This creates a potentially attractive combination: **a large underlying market, increasing healthcare spending, faster-growing adjacent categories and a fragmented distribution structure that leaves room for organised players to gain share.**

A Fragmented Customer Base Creates a Large Distribution Opportunity

The size of the market is only one part of the opportunity. **The structure of the Indian pharmacy market is equally important**. As India has more than 1 million pharmacies with roughly **97–98% still being independent stores**, highlighting how fragmented customer base remains.

Even the organised chains have limited penetration. Apollo and MedPlus together have around **10,000–12,000 stores**, while the next-largest players have significantly smaller networks. Even with the gradual rise of organised pharmacy chains, the sheer size and fragmentation of the independent pharmacy base creates a large addressable market for organised distributors like Entero



*Entero Healthcare Solutions Limited
May 26, 2026*

Prabhat Agrawal:

Yes. So the reason why we have this unique advantage is because the supplier base is also very fragmented. And the customer base is also very fragmented. And you asked specifically about the retail being unorganized in India. Retail, do you see a lot of consolidation happening in the near future?

I don't see that because India has more than 1 million pharmacy stores. The number of stores that belong to chain stores is hardly 15,000. So still 97%, 98% of store network in India is basically mom-and-pop. And even that 1%, 2% organized retail is happening after 30, 40 years.

The biggest store network in India is MedPlus, which is 5,500 and second biggest, let's say sorry, first is Apollo, second is MedPlus. Together, they add up to like 10,000, 12,000 stores across the country, and they are adding like 500 - 600 stores every year. So you imagine -- and then after MedPlus and Apollo, there is no one having even 500 stores in India, right? So consolidation on retail is not going to be such that the organized chain will start contributing a large part of India's pharmacy business, pharmacy retailing business.

The product universe is also large with Indian pharma industry has **more than 1.5 lakh SKUs**, compared with the **97K+ SKUs currently handled by Entero**. This highlights the headroom to increase both **product breadth and wallet share** with existing customers.

Shivkumar Prajapati: Understood, sir. And sir, my last question would be if you can help me with some data points. So, we are already handling more than 80,000-85,000 plus SKUs. So, what would be the total estimated SKUs in the pharma industry? And if you can provide revenue split by Tier 1, Tier 2, and beyond, if that is possible for you to share?

Prabhat Agrawal: So, in pharma industry, probably SKU could be more than 150,000.

We have established that India has a large and growing healthcare market, but for Entero, the more relevant opportunity is the **market that actually flows through the distribution channel.**

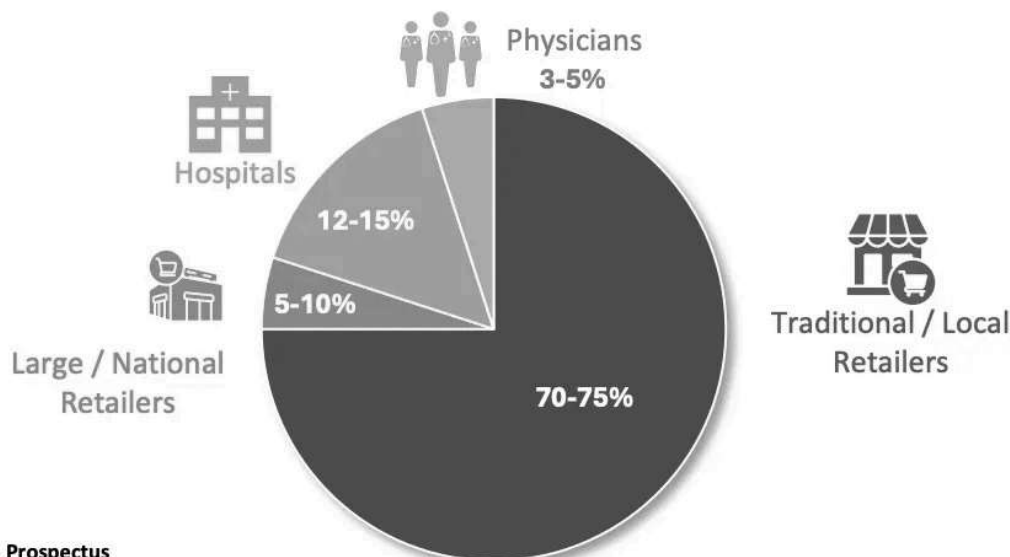
Entero's target addressable market : **pharmaceuticals plus medical devices and consumables, including hospital supplies** was valued at around **₹2.7 lakh crore (US\$33.2 billion)** in FY23 and was expected to grow at **10–11% CAGR** through FY28.

What makes this opportunity particularly interesting is not just its size, but its **fragmentation**. Around **90–92% of distributor sales were still with traditional local distributors**, while large/national distributors accounted for only 8–10%. The latter expected to increase to **20–30% by FY28**.

Large and Growing Target Addressable Market...



Industry Dominated by Traditional Local Retailers



Source: Prospectus

Note:

(1) Target Addressable Market is with respect to pharmaceutical and medical devices (including hospital supply of medical devices and consumables)

Market is highly fragmented with Entero being top 3 with ~2% market share. Also, a large portion of this market is unorganized and this is where players like Entero are playing on an acquisition led unorganized to organized shift and consolidation.

For Entero, therefore, the TAM story has **two components: the underlying market growing, and the organised distribution share is also increasing**. Entero is trying to participate in both trends by gaining market share from local distributors while expanding into adjacent categories such as MedTech and diagnostics.

Underlying Organic Growth: The Core Engine

While acquisitions have been an important part of Entero's growth, the more important number to track over the long term is **organic growth**. As the company moves towards integrating acquisitions and extracting synergies, understanding the underlying growth of the core business becomes increasingly important.

Historically, Entero has targeted **1.5–2.0x IPM growth** through new customer additions, deeper wallet share, expansion into new geographies and product categories. In FY26, organic growth was **13.4%**, or **15.6% on a like-for-like basis**, versus IPM growth of around 8–9%, implying roughly **1.6x IPM growth**.

The current growth is being driven by a few key factors:

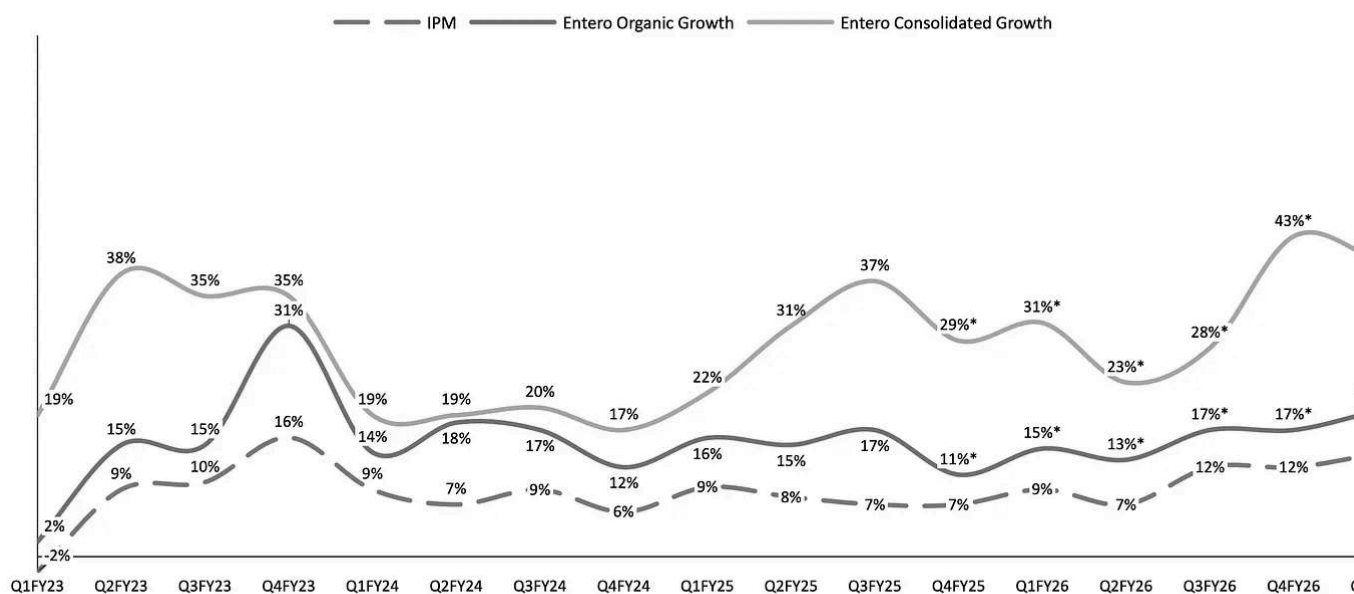
- **Adding new pharmacies and hospitals**, particularly in underserved markets
- **Increasing wallet share** from existing customers through a wider SKU offering
- **Expansion into new product categories**, including MedTech, diagnostics and trade generics
- **Geographic expansion and greater network density**
- **Cross-selling acquired businesses** through Entero's existing customer and distribution network

Management has also indicated that the underlying like-for-like growth remains around **1.5x IPM**, even as reported organic growth can be affected by accounting changes and the exit of lower-ROIC businesses.

What Could Slow This Growth?

The key risk is that Entero's outperformance over IPM may moderate as the base becomes larger. Therefore, the key metric to track is not simply consolidated revenue growth, but organic, like-for-like growth versus IPM. If Entero can consistently maintain a ~1.5x or higher growth multiple over IPM while successfully integrating acquisitions, the long-term growth thesis remains strong.

The chart below puts this into perspective by separating IPM growth, Entero organic growth and consolidated growth.



*Like-for-Like growth includes impact of revenue recorded on net margin basis and divestment of a subsidiary

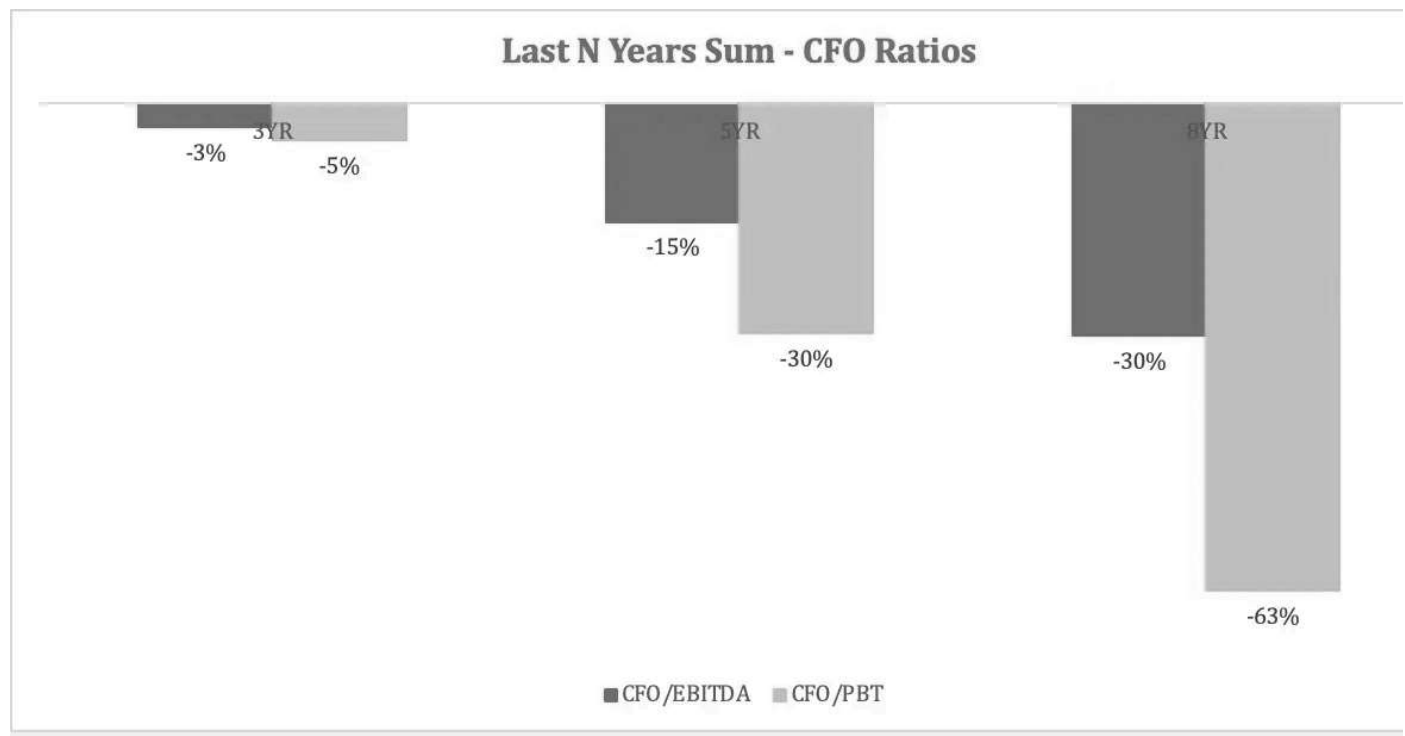
Having understood what Entero sells, how the deal economics work, and what shape the industry takes, the natural next question is: have these forces actually translated into financial performance, or is the story still aspirational? That is what Chapter 3 examines.

Chapter 4: Financial Analysis — Reading the Margin and Cash Flow Story

A first-time look at Entero's reported numbers can produce two opposite reactions.

First sees a 30% revenue CAGR and a PAT line that has gone from -15Cr in FY21 to ₹146 Cr in FY26 - a transformation.

Growth Summary						
Duration	Sales	EBITDA	EBIT	PBT	PAT	EPS
5 YR	30%	78%	91%	-275%	-249%	-144%
3 YR	26%	61%	73%	-465%	-315%	-198%
1 YR	29%	55%	30%	29%	21%	21%
TTM (Latest 4 vs PV4)	32%	63%		42%	22%	



The sceptic sees a lower margin, persistent negative operating cash flow, and a balance sheet swelling with goodwill. Both are looking at real things.

Balance Sheet

Consolidated Figures in Rs. Crores / View Standalone

CORPORATE ACT

	Mar 2020	Mar 2021	Mar 2022	Mar 2023	Mar 2024	Mar 2025	M
Equity Capital	0.10	0.10	4	4	43	44	
Reserves	-16	-32	-62	-73	1,595	1,681	
Borrowings +	603	720	974	1,101	338	385	
Other Liabilities +	162	147	210	275	369	593	
Total Liabilities	749	836	1,125	1,308	2,345	2,703	
Fixed Assets -	186	189	262	268	289	559	
Land	0	0	0	0	0	5	
Building	13	14	15	15	16	20	
Plant Machinery	5	6	10	10	11	16	
Equipments	4	5	5	7	7	10	
Computers	3	4	6	7	8	10	
Furniture n fittings	20	22	26	29	30	35	
Vehicles	2	2	2	3	4	7	
Intangible Assets	89	89	150	167	193	424	
Other fixed assets	65	77	97	102	115	159	

The financial story needs to be looked at in context. Entero has changed significantly over the last few years through acquisitions, rising scale, margin improvement and increasing investments in its distribution network.

So rather than looking at one year in isolation, it is important to understand what has changed over time and how those changes have flowed through the P&L, balance sheet and cash flow.

Part 1: Margin decomposition — what is driving the lift

Entero's gross margin has improved meaningfully over the years, reaching 11.4% in FY27, up 147 bps YoY. Management attributes the recent expansion mainly to three factors: scale-led procurement benefits, a higher contribution from MedTech, and the exit of certain low-margin businesses.

Profit & Loss

Consolidated Figures in Rs. Crores / View Standalone

HEATMAP

RELATED PARTY

PROJECT

	Mar 2020	Mar 2021	Mar 2022	Mar 2023	Mar 2024	Mar 2025	Mar 2026
Sales +	1,350	1,773	2,522	3,300	3,922	5,096	6,591
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Operating Profit	23	15	24	64	112	172	266
OPM %	1.7%	0.8%	1.0%	1.9%	2.9%	3.4%	4.0%

- **Procurement scale:** Higher purchase volumes are improving buying economics and manufacturer terms.
- **Higher-margin categories:** MedTech is becoming an important structural driver where Entero plays a larger commercial role rather than just fulfilment. Management expects MedTech revenue to cross **₹1,000 crore in FY27**, with higher gross and EBITDA margins than core pharma distribution.
- **Exit from low-margin business:** Entero is consciously giving up certain low-margin/low-ROIC accounts and reallocating resources towards better-return opportunities.

Quarterly Summary							
KPI	7QB	6QB	5QB	4QB	3QB	2QB	CQ
TTM Sales	4791	5096	5403	5673	6020	6591	7128
TTM OP	152	172	191	211	229	266	313
TTM PBT	115	139	148	158	159	179	210
TTM OPM	3.2%	3.4%	3.5%	3.7%	3.8%	4.0%	4.4%
TTM PBT Margin	2.4%	2.7%	2.7%	2.8%	2.6%	2.7%	2.9%
Quarterly OPM	3.7%	3.7%	3.6%	4.0%	4.0%	4.5%	5.0%
Quarterly PBTM	2.9%	2.8%	2.6%	2.8%	2.4%	3.0%	3.5%
YoY Sales Growth				20.8%	25.6%	42.6%	38.2%
YoY OP Growth				46.2%	35.5%	75.9%	93.7%
YoY PBT Growth				28.3%	2.4%	55.7%	84.6%

The improvement in EBITDA is broader than just gross-margin expansion. **Operat leverage is now becoming an additional driver** as revenue grows faster than the underlying cost base. Management has indicated that at around **20% revenue growth** the cost base should not increase at the same pace, creating further EBITDA levera

This is already visible: **EBITDA margin reached 5% in Q1 FY27**, and management guided for **5% for FY27**, with its internal aspiration being **above 6%** over time.

Gross margin expanded 147 basis points year-on-year to 11.4%, and EBITDA margin expanded 143 ba points to 5%, while EBITDA growing 94% year-on-year, nearly two and a half times our revenue growth ra This margin expansion was driven by three levers: scale-led procurement economies, a growing share revenue from the MedTech business, where we play a full commercial role rather than a pure fulfillment ro and the deliberate exit from certain low-margin accounts. Having achieved a 5% EBITDA margin in the fi quarter itself means we are already tracking well at our full-year FY27 guidance, and our focus for t upcoming quarters will be on sustaining and where possible building further on this margin base.

Source: Q1FY27 Concall

In simple terms, the margin story is becoming a combination of buying better, selling a better mix, exiting unattractive business and spreading the fixed cost base over a larger revenue pool.

Our MedTech segment will continue to be the most important structural lever in our medium-term margin story. MedTech revenue is on track to cross 1000 crores in FY27 on an organic basis and continues to carry higher gross and EBITDA margins than our core pharmaceutical distribution business. Our focus for the year is deepening these existing relationships and building organic capability in this segment.

Prabhat Agrawal (Managing Director and CEO)

So when I said 20%, I said 20% total growth in business, you know, we should, and without any meaningful acquisitions, there's some small acquisitions might happen in that 20%. And of course as we grow 20% cost base itself is not going to grow at 20% so you will gain something from operating leverage and at same time you know the margin expansion at gross margin also would happen because of the scale that are getting the procurement benefits that we get as we grow, right. So of course, you know, at a 20% growth rate over next three four years, you know, there should be a good expansion on EBITDA margins also.

Alok Dalal (Jefferies)

Yeah, because from 5%, the ask rate is only 30 basis point every year which I believe should be achievable. So 6% not a tall ask, is what I'm trying to see.

Prabhat Agrawal (Managing Director and CEO)

Why should we limit, why should we limit to 6% only?

Alok Dalal (Jefferies)

No, no, so 6%, let's say easily achievable is what I'm trying to say.

Prabhat Agrawal (Managing Director and CEO)

Yes, yes. Our internal aspiration and target would be more than 6%, for sure.

Source: Q1FY27 Concall

This also ties back to the global examples discussed earlier: **working-capital discipline, higher-margin/value-added businesses, technology-led efficiencies and operating leverage** can all contribute to better economics as a distributor scales. Entero is now attempting to build these levers simultaneously.

Part 2 : Cash flow — the contrast that confuses

Despite EBITDA increasing from ₹22 crore in FY21 to ₹266 crore in FY26, Entero generated negative operating cash flow in each of the previous years. The key reason was **working-capital absorption** as the business expanded, higher inventory and receivables required cash upfront, while the benefits of the additional revenue and scale were realised over time.

Ratios

Consolidated Figures in Rs. Crores / View Standalone

HEAT

	Mar 2020	Mar 2021	Mar 2022	Mar 2023	Mar 2024	Mar 2025	M
Debtor Days	63	50	54	57	57	59	
Inventory Days	57	54	49	41	43	52	
Days Payable	32	22	22	25	24	31	
Cash Conversion Cycle	88	82	81	73	77	80	
Working Capital Days	57	54	41	32	48	65	
ROCE %		1%	1%	5%	7%	9%	

This made cash generation look weaker than the underlying improvement in profitability. However, **FY26 marked an important inflection point**, with operating cash flow turning positive at ₹96.2 crore attributed **improving net working-capital days and higher EBITDA margins**

Annual OCF



Two structural changes also help. First, the GST rate change from 12% to 5% on certain pharma products (effective Q3 FY26) lowers GST receivable balances and improves OCF by an estimated 3–5 days of working capital. Second, the slowdown the pace of new acquisitions reduces the working capital absorption lump.

Kumar Saurabh (Scientific Investing)

Hi Prabhat. Congrats on a great set of numbers. My question is around, I believe we have reached to a point where for the incremental 20% plus growth. Whatever is the incremental working capital required, remember your guidance, we want to convert 50 percentage of EBITDA into OCF, which gives us somewhere around 200 crores.

So my sense is we are at a point where because of our margin improvement, we should be able to take care of the incremental working capital through our internal accruals. And if we are able to do that, it means we will be able to convert more percentage of EBITDA to PBT. And already we have done great. I think two years back we were 33% PBT to EBITDA. Now we are around 60%. So do you see next two, three years possibility? First, that we will not need an incremental debt if we are not going for more acquisition, and second, the possibility to reduce this debt over a period of time if we are not going for incremental acquisition, and how do you see going forward in a long term? Do you see again we will come back to, you know, acquisition mode or you will be more interested in paying the debt and improving your PBT to EBITDA ratio, which will ultimately again give us an operating leverage on the profitability size versus sales growth rate?

Management is targeting **50% EBITDA-to-OCF conversion in the coming years**. As margins improve and working-capital efficiency increases, the business should increasingly be able to **fund its incremental working-capital needs through internal accruals rather than relying on additional debt** as Current Total Debt stands at 677 as of FY26.

The resulting cash flow also creates flexibility in capital allocation. Which can be deployed towards **acquisitions, debt repayment or buying out minority interests** depending on which option offers the best incremental return.

Prabhat Agrawal (Managing Director and CEO)

Yeah. So first of all, let me clarify when I say OCF. When I say OCF, OCF is after accounting for changes working capital, right? So when I say 50% of EBITDA getting converted to OCF, means EBITDA minus working capital changes whatever is the remaining is supposed to be 50% of EBITDA. Okay. So, you know, OCF is positive because your OCF positive means you have enough internal accrual to fund your working capital changes. Okay. Now, if we are saying that in this year we will have 50% left after accounting for working capital changes that money could be used for acquisition, it could be used for paying off debt, it can be used for buying minorities. There are multiple uses of that extra cash flow that we have, we need to see and compare what gives us the most incremental improvement in EPS, accordingly we will deploy that money.

Therefore, the next phase of the story is not just about growing EBITDA, but converting a larger portion of it into cash and using that cash to improve the balance sheet and fund future growth. Whether this trajectory continues depends, more than anything, on whether the management team has the discipline to execute. That is the subject of Chapter 4

Chapter 4: Management & Execution

The promoters and the founding hypothesis

Entero was founded in January 2018 by Prabhat Agrawal (Managing Director and CEO) and Prem Sethi (Whole-time Director and COO).

Prabhat Agrawal is a chartered accountant and chartered financial analyst, with a master's in management from the Indian School of Business, Hyderabad. His prior career was as CEO of Alkem Laboratories one of India's larger domestic pharma companies and before that, Group CFO at Metalfrio Solutions in Brazil and Deputy Operations Director at Frigoglass Industries. He was named 'CEO of the Year' at the 9th Annual Pharmaceutical Leadership Summit in 2016.



Prabhat Agrawal ✓ · 3rd

Co-founder and MD at Entero Healthcare | CA, MBA (ISB) and CFA Charterholder

Mumbai, Maharashtra, India · [Contact info](#)

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About

Experienced business leader with a demonstrated history of strategic and executional strength in the Healthcare, Industrial and Financial Services Industry. Skilled in Business Strategy, Management, Operations, Financial Reporting and Corporate Finance. Strong management professional with Chartered Accountant, MBA and CFA charter holder qualifications.

Experience



Co-founder and Chief Executive Officer

Entero Healthcare

Apr 2018 - Present · 8 yrs 5 mos

Mumbai Metropolitan Region



Chief Executive Officer

Alkem Laboratories Ltd.

Oct 2014 - Mar 2018 · 3 yrs 6 mos

Mumbai Metropolitan Region

Established in 1973 and headquartered in Mumbai, Alkem (NSE: ALKEM, BSE: 539523, Bloomberg: ALKEM.IN, Reuters: ALKE.NS) is a leading Indian pharmaceutical company with global operations,... [more](#)



Group CFO

Metalfrio Solutions SA

Nov 2012 - Oct 2014 · 2 yrs

Sao Paulo, Brazil

Metalfrio Solutions S.A. (Bovespa: FRIO3) is one of the world's leading companies in plug-in commercial refrigeration equipment with a product portfolio that includes vertical and horizontal plug-in... [more](#)



Frigoglass

5 yrs

Regional Finance Head - Asia Pac and Africa

Apr 2012 - Nov 2012 · 8 mos

New Delhi, India

A public listed company (Athens Stock Exchange) incorporated in Greece with production facilities and sales offices spread in 20 countries across five continents and products sold in more than 100... [more](#)

Deputy Operations Director

Aug 2011 - Mar 2012 · 8 mos

Lagos, Nigeria

The company is a subsidiary of Frigoglass SAIC. It has a turnover of 100 Mn Euro with 1400 people in employment and is listed in the Nigerian Stock Exchange. It operates the 2nd largest container... [more](#)

Finance Director

Dec 2007 - Aug 2011 · 3 yrs 9 mos

Lagos, Nigeria



Senior Manager - Corporate Strategy and Business Development

Aditya Birla Group

Mar 2007 - Dec 2007 · 10 mos

Prem Sethi has a bachelor's in pharmacy and a master's diploma in clinical research and pharmacovigilance. His prior experience was at IQVIA Consulting (Director — Offering Development and Product Management) and Information Services India. Where Agrawal brings the operational and financial track record from Alkem, Seth brings deep visibility into pharma data, manufacturer pain points, and the commercial side of the supply chain.



Prem S.
Co-Founder Entero Healthcare

Experience



Co-Founder and COO

Entero Healthcare Solutions Private Limited · Self-employed

May 2018 - Present · 8 yrs 4 mos

Mumbai Metropolitan Region



QuintilesIMS

2 yrs 8 mos

Mumbai, Maharashtra, India

- **Director - New Offerings and Product Management- South Asia**

Apr 2017 - May 2018 · 1 yr 2 mos

- Manage new products development at QIMS
- Create new solutions for ever-changing healthcare industry
- Create BI solutions based on QIMS Data Assets.
- Manage Data Collection Field Team
- Manage Production Environment for QIMS
- Manage Reference Data Teams at QIMS

- **Head-New Offerings & Supplier Services-South Asia**

Oct 2015 - Mar 2017 · 1 yr 6 mos



Head Operations South Asia-GTO

IMS Health

Sep 2013 - Sep 2015 · 2 yrs 1 mo

Mumbai, Maharashtra, India

- Manage Production Environment for India
- Manage Data Collection-Pan India... more



Deputy General Manager-II

Cognizant Technology Solutions

Aug 2012 - Sep 2013 · 1 yr 2 mos

Hyderabad, Telangana, India

- Manage Client Engagements.
- Drive client SLA's.... more



Informa Business Information--Datamonitor



4 yrs 2 mos

Senior Practice Leader Healthcare (MedTRACK)

Aug 2010 – Jul 2012 · 2 yrs

Greater Hyderabad Area

The acquisition track record — discipline as observed, no asserted

There are several ways to evaluate whether a roll-up's M&A track record is high quality. The most informative are

- (a) the multiples paid, and
- (b) the source of funding,

On multiples paid. The disclosed range across 50+ acquisitions has remained mid-to-high single-digit EV/EBITDA. Critically, multiples have not inflated as Entero's own valuation has risen: the targets are local, family-owned distributors, not strategic competitors with market power, so price discipline is sustainable as long as Entero remains the most credible institutional buyer in the segment.

Post-acquisitions, growth came from product portfolio expansion, customer base expansion (cross-sell into Entero's network), and warehousing/operations leverage.

On source of funding. The early acquisitions (FY18–FY23) were funded by primary capital from OrbiMed and other PE rounds, plus modest debt. Post-IPO, the deployment has been from net IPO proceeds (₹237 Cr earmarked for inorganic) plus internal accruals.

What management has consistently said versus what they have delivered

Across earnings calls since the IPO, management has provided a fairly consistent set of forward-looking commitments: 1.5-2x of IPM Growth, EBITDA margin expansion, working capital improvement and OCF turning positive on a full-year basis in FY20

Full-year trajectory has been consistent with what they have guided for as shown in the below “SI Walk the Talk Analysis”

FY24 (IPO year)

Fiscal Year	Management Guidance	Actual Outcome	Indicator	Performance Trend
FY24	Key messages (IPO RHP, AR, Q3/Q4 concalls): grow faster than IPM , maintain ~30% CAGR over FY20–24, expand EBITDA margin vs 1.94% , improve ROCE, use IPO proceeds exactly as disclosed, and prepare for 35–40% growth & +100 bps margin expansion in FY25 . No explicit numeric FY24 target, but clear directional promises.	Revenue ₹39,223 mn (↑19% vs FY23) vs IPM 7.6% → growth 2.5x IPM , though below historic ~30%; EBITDA ₹1,118 mn , margin 2.85% (+91 bps); PAT ₹398 mn (loss to profit); ROCE ~10.2% (from 6.1%); net WC 67 days (slight increase vs 64). IPO fresh issue ₹1,000 cr; monitoring agency confirms “ <i>no material variation</i> ” vs objects; debt repaid, company net-cash at FY24.	●	Delivered & Improving (with growth moderation) – They did what they said directionally: outgrew IPM, expanded margins by ~100 bps, strengthened ROCE, used IPO money exactly as promised. The only “soft spot” is revenue growth moderating to ~19% vs the historic 30% narrative, but that was explicitly linked to a deliberate pause in M&A pre-IPO.

Fiscal Year	Management Guidance	Actual Outcome	Indicator	Performance Trend
FY25	Very explicit in Q4 FY24 concall & FY25 AR: – Revenue growth 35–40% (~20% organic, rest inorganic from ₹1,000 cr+ acquisitions). – EBITDA margin expansion of ~100 bps (from 2.9% → ~3.9%). – Turn H2 OCF positive; move towards positive full-year OCF over time . – Maintain WC days broadly similar (mid-60s) with modest improvement; all acquisitions margin-accretive (6–8% EBITDA) and in new geographies / segments.	Revenue ₹50,958 mn (↑30% vs FY24). On like-for-like basis (adjusting for net-margin accounting) they still talk about ~30% – below the 35–40% band by 5–10 ppt → clear miss numerically. EBITDA ₹1,715 mn , margin 3.37–3.40% (vs 2.9%) → +50–60 bps, not full +100 bps . PAT ₹107 cr (↑170%); ROCE 10.7% ; ROE 7.7% . Net WC 70 days (vs 67; slightly worse). Operating cash flow –₹76.9 cr for the year, but H2 OCF turned positive , as MD emphasises. M&A: 10 acquisitions closed, contributing ~₹792 cr ARR; announced 6 more (~₹400 cr). All positioned as margin-accretive ; pro-forma EBITDA uplift visible.	● (clear shortfall vs numerical guidance, but strong directional delivery)	Almost Met & Improving but Below Stated Band – The company missed its own quantitative band on revenue growth and EBITDA uplift and failed to turn full-year OCF positive, though it did hit H2 OCF and margin-accretive M&A. Strategically, FY25 was a “landmark year” (₹5,000 cr revenue, ₹100 cr+ PAT), but relative to the aggressive IPO/FY25 guidance they under-delivered .

Fiscal Year	Management Guidance	Actual Outcome	Indicator	Performance Trend
FY26	At the start of FY26 (AR, Q1 & Q2 presentations; reiterated in AR FY26): clear 3-point guidance: 1) ~30% like-for-like revenue growth , 2) EBITDA margin 4% , 3) positive operating cash flow ~₹100 cr . They also promised further WC improvement (towards ~60 days) , ROCE & ROE uplift, and to build MedTech to >₹1,000 cr annualised by FY27 (later accelerated to FY26 itself).	Revenue ₹65,912 mn (↑29.3% reported; 31.5% like-for-like). EBITDA ₹2,659 mn , EBITDA margin 4.03% (from 3.4%). PAT ₹145.8 cr (↑36%). Gross margin 10.3% (vs 9.5%). ROCE 14.6% (vs 10.7%), ROE 12.5% . Net WC days: year avg 68 (vs 70) and Q4 run-rate 59 days . Operating cash flow ₹96.2 cr , very close to the “~₹100 cr” commitment. MedTech annualised revenue crossed ₹1,000 cr (~15% of revenue) as promised, with MedTech M&A adding 70–90 bps to gross margin and 50–75 bps to EBITDA margin on a pro-forma basis.	●●	Overachieved & Improving – On the trio of growth, margin, and cash flow , FY26 is a clean delivery year : they essentially hit or slightly exceeded all three targets on a like-for-like basis and pulled forward the MedTech ₹1,000 cr milestone. ROCE and ROE re-rated sharply, and WC efficiency improved to high-50s by Q4. This is the clearest example of management “walking the talk” against explicit numbers.

Having now mapped the business model, the unit economics, the industry, the financial trajectory, and management discipline, the work that follows is to state clearly what has to true for the thesis to play out, and what could break it. That is Chapter 5.

Chapter 5: Thesis vs Anti-Thesis

The Entero thesis has a simple shape and a complicated execution requirement.

The shape: a fragmented industry will consolidate; the most credible institutional consolidator will capture disproportionate value; a margin transformation is layered on top of the volume story.

The complication: every component has a real failure mode, and the thesis fails if any one of them breaks. What follows is a tight statement of the bull and bear cases.

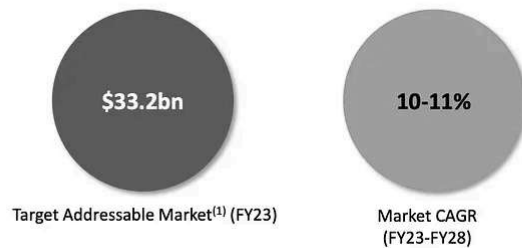
The Thesis

1. **The consolidation:** India's top three distributors have moved from less than 5% share pre-2015 to 8–10% today, with CRISIL and management projecting 20–30% by FY28. The structural drivers : GST, capital availability, technology adoption, regulatory tightening, manufacturer-side preference for scaled partners. Importantly, this is not a 'India will become like the US' thesis. The US top-3 share is 90–95%, which is not the realistic bound for India in any reasonable horizon. The realistic bound is the 20–30% that CRISIL forecasts or even 15% at that alone means the organised distribution pool roughly triples.

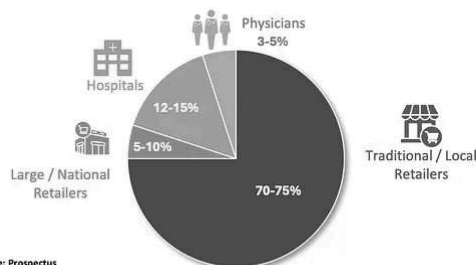
Healthcare Products Distribution Market Overview



Large and Growing Target Addressable Market...



Industry Dominated by Traditional Local Retailers



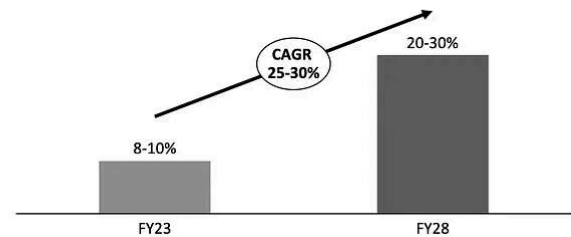
Source: Prospectus

Note:

(1) Target Addressable Market is with respect to pharmaceutical and medical devices (including hospital supply of medical devices and consumables)

...Led by Increasing Share of Large / National Distributors

Share of the total distributor sales for Large / National pharmaceutical distributors



Key Trends

- Consolidation in the distribution industry
- Technological advances to support operational efficiencies in distribution
- Micro-market and regional data opportunities
- Higher demand for surgical and medical devices
- Increasing government focus on generics to further support distributors
- Omnichannel adoption to improve demand for pharmaceutical distribution

2. The MedTech pivot is a margin transformation, not just a growth lever.

MedTech

FY27 revenue >₹1,000 cr (guided)

Share of total ~12-13%

Gross margin materially above core

Growth target 20% organic, 3-5 yrs

Role Generation + fulfilment

The designated margin lever. Built in 18 months through three acquisitions — Anand Chemiceutics (IVD, capital equipment, applied genomics: oncology / infectious disease / rare disorders; 1,500+ customers), Cardiopathy (cardiovascular) and Bioaide (interventional cardiology, ENT, wound management, neuro-psychiatry). Principals include Abbott, Medtronic, Terumo, MED-EL, Asahi, BrainsWay, Sechrist.

Why the margin is structurally better: MedTech distribution has far fewer credible national players than pharma, so Entero can sign pan-India exclusive arrangements and take a commercial role — end-to-end demand generation plus fulfilment — rather than pure logistics. Fewer competitors, deeper contracts, rich economics.

Sub-segment focus: IVD (17% of the Indian MedTech pool) and Cardiology/Orthopaedics (15%). Indian medical devices market ≈ US\$14–16bn growing 12–14% p.a. — a market growing faster than IPM, where Entero has ~0.5% share. That is the single largest runway on the page.

3. **Operating leverage runway.** EBITDA margin moved from 1.0% in FY22 to 4.0% FY26 and 5% in Q1FY27. Management has guided 6%+ in coming years. The shift to MedTech and value-added services is the principal driver going forward. OPEX leverage continues but asymptotes.
4. **Working capital improvement is becoming a structural cash unlock.** Entero's working-capital efficiency has been improving, with net working-capital days declining from earlier levels as the company focuses on inventory and receivables.

This improvement, combined with higher EBITDA margins, helped operating cash flow turn positive at **₹96.2 Cr in FY26**, versus negative OCF in every previous year.

5. Management is now targeting **~50% CFO-to-EBITDA conversion**. At the current EBITDA base, this implies a meaningful increase in internally generated cash that can fund incremental working capital and potentially support **debt repayment, acquisitions or minority buyouts** supporting improvement in ROCE over time

6. Core Pharma Distribution

Core pharma distribution

Est. FY27 revenue **~₹5,400-5,800 cr**

Share of total **~66-70%**

Gross margin **~9-10% (est.)**

Growth driver **IPM + 5.5pp**

Role **Demand fulfilment**

The base load. Branded generics and patented drugs moved from 3,000+ manufacturers to 72,000+ retail pharmacies through 138 warehouses across 475 districts. Its economics are the two-way network effect CEO describes: Entero has manufacturers because it has customers, and customers because it has manufacturers.

How it scales: wallet share, not door count. A retailer typically splits purchases across several distributors. Entero's fill rate, SKU range (83,400) and one-stop convenience pull that split its way. Margin here expands only through procurement scale — the CEO confirmed pharma gross-margin gains are "primarily driven by scale, driven procurement efficiencies."

Ceiling: the prospectus itself puts the distributor margin band at 8-15% gross. At 11.4% blended, further core-pharma expansion is incremental.

The Anti-Thesis

1. Roll-up integration risk

Entero's growth relies partly on successfully integrating acquisitions and extracting synergies. If acquired businesses fail to reach targeted margins, or integration costs and working-capital requirements remain elevated, the expected earnings and ROCE improvement could fall short.

2. IPM growth could moderate

Entero's organic growth has historically been well ahead of IPM, with management targeting **1.5-2x IPM growth**. If IPM settles at a lower growth rate, Entero's organic growth could also moderate. Combined with a slower pace of acquisitions, this could bring overall growth below the levels currently expected.

3. Working capital remains a key risk

Distribution is inherently working-capital intensive. Any deterioration in

inventory turns, receivables or credit terms can absorb cash quickly. FY26's positive OCF is encouraging, but **sustaining cash conversion through different growth and credit cycles remains an important test.**

4. **Margin expansion may have a ceiling**

Entero has already reached a **5% EBITDA margin**, with management aspiring to move above 6%. However, pharma distribution remains structurally low-margin. Further expansion will depend on procurement benefits, operating leverage and rising contribution from higher-margin MedTech/commercial businesses.

5. **Organic growth needs to remain strong**

As the revenue base becomes larger, maintaining **1.5–2x IPM growth** will become progressively harder. New customer additions, higher wallet share, geographic expansion and cross-selling therefore need to continue contributing meaningfully. A slowdown in these underlying engines would make the business increasingly dependent on acquisitions to sustain growth.

6. **Minority Interest: Improving, but Still Relevant**

Minority interest remains an important factor in understanding earnings attributable to Entero shareholders. In **Q4 FY26, minority interest was ~38% of PAT**, meaning a significant portion of consolidated profits was not attributable to Entero.

However, management expects this to **decline to around 25–27% of PAT in FY27** supported by increased ownership and integration of subsidiaries.



*Entero Healthcare Solutions Limited
May 26, 2026*

Prabhat Agrawal:

So what we can say for next year, you can take around 25% to 27% of PAT before minority interest to be contribution from minority interest. This quarter, the minority interest was almost 38% of our PAT. That will be reduced by 10% to 12% in the next year.

These observations set up the framework for valuation, but valuation itself requires explicit scenarios. That is the work of Chapter 6.

Chapter 7: Valuation Framework

Entero today is a very different business from what it was a few years ago. Revenue scaled , EBITDA margins have expanded from around 1% to 5%, operating cash flow has turned positive, and the company is increasingly moving towards higher-margin MedTech and value-added distribution.

For our valuation, **We have deliberately kept the framework conservative and have built a base, bull and bear framework** around the key operating variables revenue growth, EBITDA margins, cash conversion and minority interest.

Our base case assumes ~22% consolidated revenue growth, a 5.0% EBITDA margin in FY27 improving to 5.2% in FY28, and 50% CFO/EBITDA conversion. These assumptions are broadly aligned with management's current guidance, which will take EBITDA from 264 in FY26 to 383 in FY28 (Attributable to Entero)



Entero Healthcare

Assumptions	Bear	Base	Bull
Consolidated Revenue Growth	20%	22%	24%

Bear Case	FY25	FY26	FY27	FY28
Revenue	5096	6591	7909	9491
EBITDA Margin	3.4%	4.0%	4.8%	5.0%
EBITDA	172	264	380	475
Base Case	FY25	FY26	FY27	FY28
Revenue	5096	6591	8041	9810
EBITDA Margin	3.4%	4.0%	5.0%	5.2%
EBITDA	172	264	402	510
Bull Case	FY25	FY26	FY27	FY28
Revenue	5096	6591	8173	10134
EBITDA Margin	3.4%	4.0%	5.0%	5.5%
EBITDA	172	264	409	557

CFO	-77	96	201	255
	50% CFO/EBITDA Conversion (Base Case)			
	Minority Share			
EBITDA Attributable to Entero (Minority interest expected to be ~25-27% of PAT before minority interest)	12.0%	21.0%	25.0%	25.0%
Bear Case	152	208	285	356
Base Case	152	208	302	383
Bull Case	152	208	306	418

Prabhat Agrawal (Managing Director and CEO)

So the full year guidance that we gave for this year for growth is 23% and that included like 11 or 12% from calendarization of last year's acquisition and rest was organic.

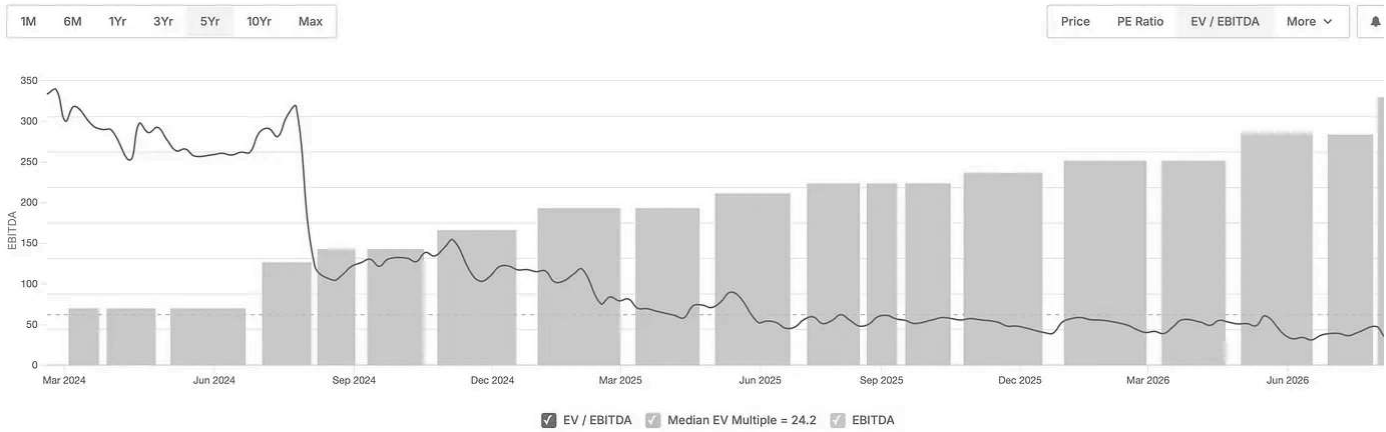
Rashmi Goyal (Arihant Capital)

Okay. So my next question is related to EBITDA margin exit rate targeted for FY27. Is 5% the floor or should the model build in further sequential expansion through the end of the last quarter of FY27?

Prabhat Agrawal (Managing Director and CEO)

So we are always targeting, you know, higher margins quarter on quarter. But for a full year guidance based on you know, we still maintain 5% as I told before that after quarter two, I might come out with, with a different number if I see if I see that second based on the first half performance I see that second half performance could be better than our guidance.

To keep the valuation framework open, what do you think Entero deserves today? What EV/EBITDA multiple would you assign to the business based on the transformation we have discussed so far?



POLL

What EV/EBITDA multiple do you think Entero deserves today?



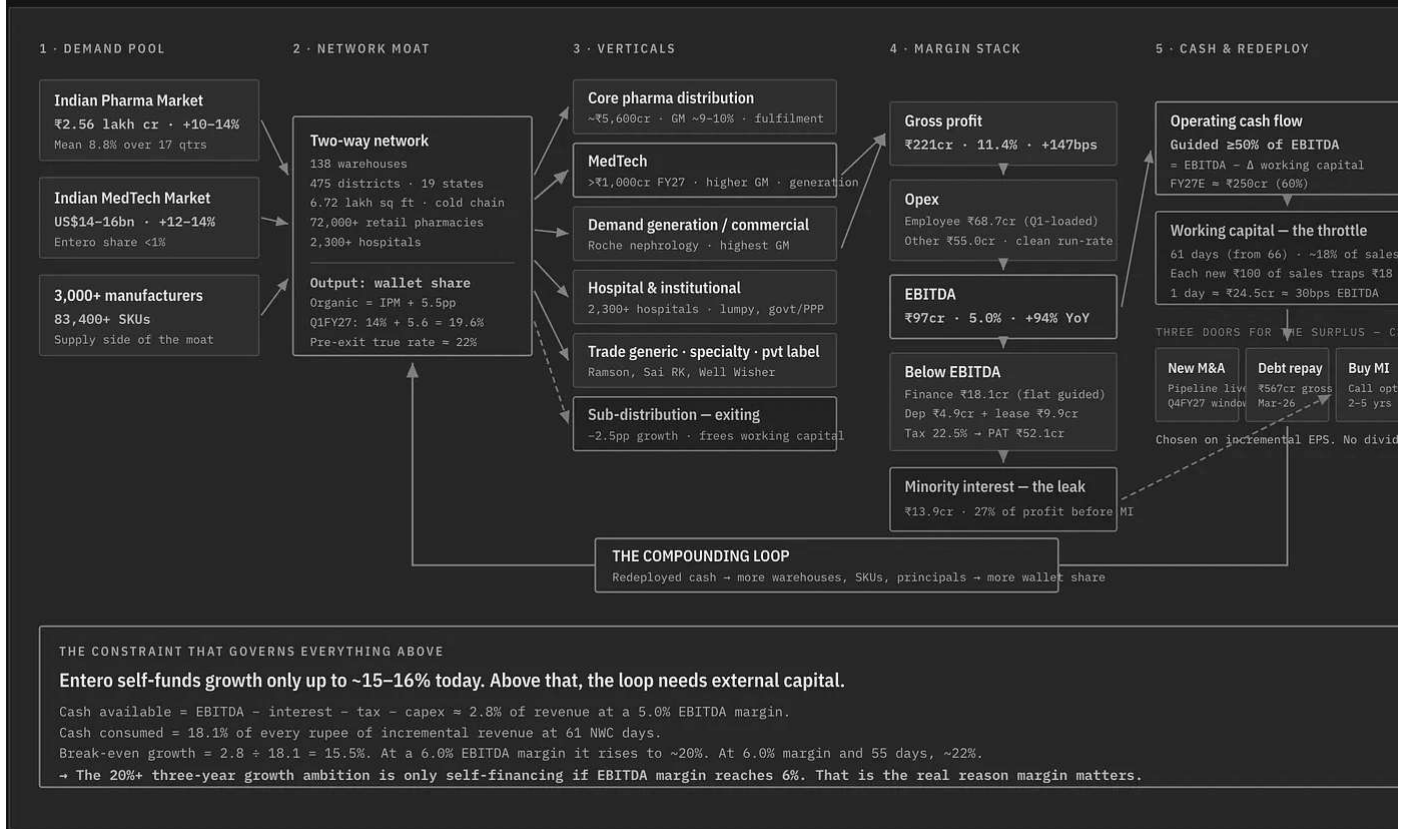
Closing Thoughts

Entero’s story is no longer just about pharmaceutical distribution. It is about **building scale in a fragmented market, improving margins, expanding into higher-value categories and converting that growth into cash and better returns.**

The opportunity is large, but execution remains key. **If Entero can sustain organic growth above IPM, integrate acquisitions effectively, improve cash conversion and move margins beyond 5%, the business can evolve into a much stronger healthcare distribution platform.**

The next few years will determine whether this transformation becomes a **sustainable compounding story or simply a period of rapid growth.**

Read left to right: the demand pool feeds the network, the network converts into six verticals, the verticals set the margin, the margin funds working capital, and the surplus returns to the top of the loop through one of three doors. The single leak in the system is minority interest.



If execution breaks, the outcome is a low-margin distribution business with goodwill on the balance sheet, slowing growth, and structural cash flow constraints. That is the failure case, and it is real.

Scorecard

Particular	Summary insights	Rating
Total Addressable Market (TAM)	In healthcare, distribution space is Rs 3L Cr TAM space growing at 8-10%. Market is highly fragmented with Entero being top 3 with ~2% market share. Also, 90% of this market is unorganized and this is where players like Entero are playing on acquisition led unorganized to organized shift and consolidation	Excellent
Industry structure	One of the most fragmented large pharma distribution markets globally. Top 3 players account for only 8-10% vs 90%+ in developed markets. Consolidation is supported by scale, GST, technology, compliance and increasing manufacturer preference for larger distributors.	Very High Tailwind
Business quality	Asset-light and locally network-driven model with low warehouse capex. SKUs and manufacturers create breadth, while city-level density strengthens the retailer-manufacturer network effect. Working capital remains the key structural constraint.	Good, Improving
Financial economics	5Y CAGR of 30% on topline. EBITDA margin improved to 4.0% . ROCE improved, while OCF turned positive and Q1 FY27 EBITDA margin reached ~5%.	Improving
Margin expansion	Margin expansion is being driven by scale-led procurement, higher-margin MedTech/value-added categories, exit from low-margin accounts and operating leverage . Management sees further margin expansion beyond the 5%	High
Growth prospects	Organic growth is expected to remain above IPM, while MedTech is expected to cross ₹1,000 Cr revenue . The key long-term driver is the combination of organic growth + selective M&A + higher-value categories	High
Management quality	Strong operating understanding of pharma distribution, with a clear expansion, margin improvement and working-capital discipline.	Good
Key risks	M&A integration, slower IPM growth, working-capital intensity, minority interest, leverage and a potential margin ceiling remain the key risks. The business must prove that higher margins and cash conversion can be sustained as the acquisition base gets larger	Real, Manageable
Valuation framework	To each his own	-

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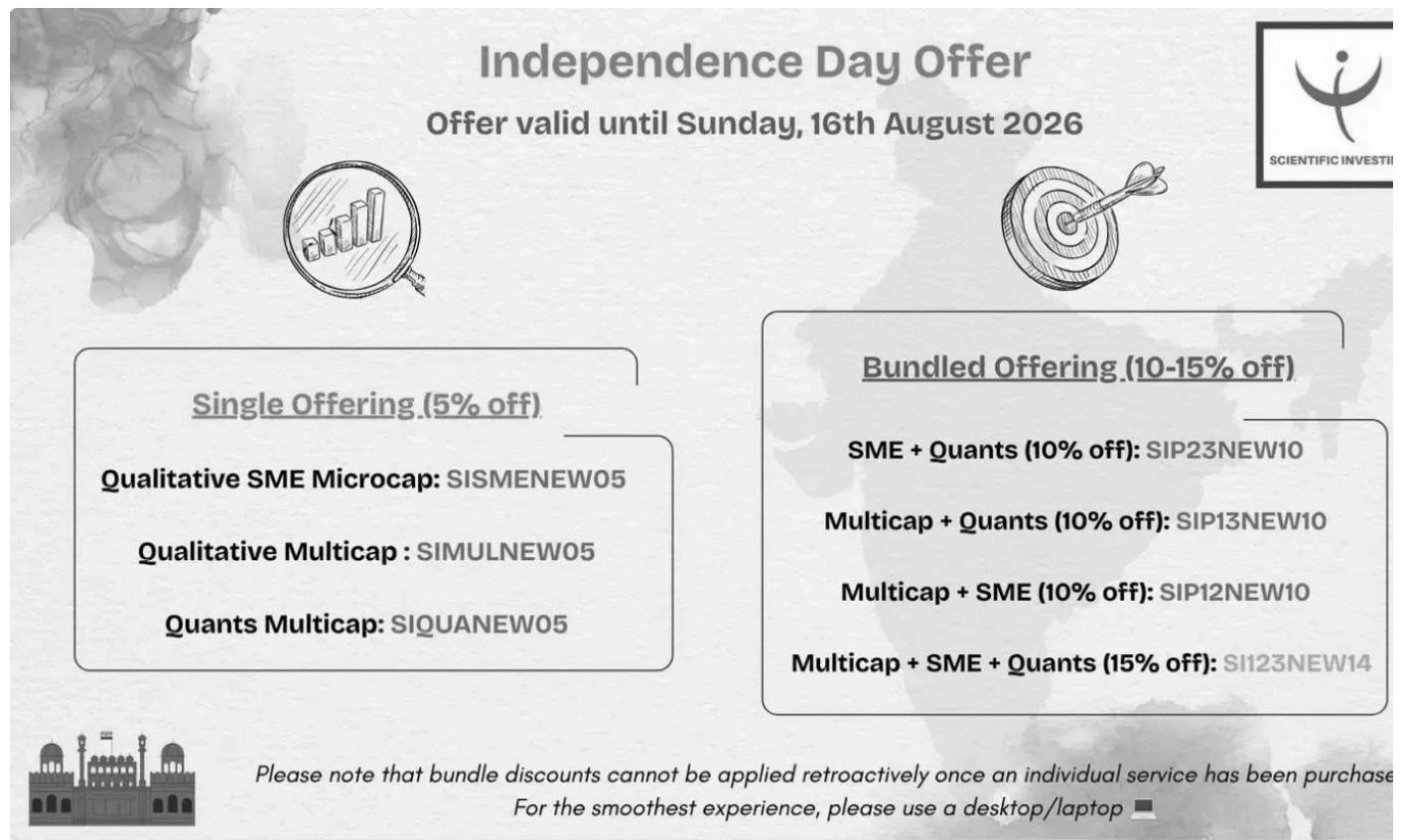
kaushik.keerti29@gmail.com

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- Conviction **Rating, Allocation & Entry-Exit** Framework
- Detailed Research Report (20-40 slides) + **Excel Model + Video** Thesis
- Quarterly Updates & Reviews
- Risk Profile: Aggressive

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Coverage: Below ~ Rs 4000Cr. Mcap

- 15-20 SME focused Research Report / Year
- Conviction **Rating, Allocation & Entry-Exit** Framework
- Detailed Research Report (20-40 slides) + **Excel Model + Video** Thesis
- Half-Yearly Updates & Reviews
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- Technofunda Momentum Based
- Rule based Entry Allocation & Exit
- Periodic portfolio rebalancing
- Risk Profile: Aggressive

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- Goal-based asset allocation
- Periodic Reviews
- Advisory Coverage: Direct Equity, ETFs/MF's, Debt, Others
- Risk Profile: Very Conservative to Very Aggressive as per Client Risk Profile

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BSE Enlistment No : 2374

Registered Office: 4 A Street, Ground Floor, Muneshwara Temple, Frazer Town Bengaluru - 560005.

Register Interest



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A guest post by
Mayank Agarwal
Financial Analyst

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Gopikrishna Reddy 15 Aug

♥ Liked by Scientific Investing

Excellent write up Mayank 🙌👏

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Sujay Rainchwar 15 Aug

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Amazing analysis man.

Well done.

Just out of the box - in case the Entero App becomes open for B2C as in for public, it will create unbelievable wealth for the company but thats too far fetched.

The huge retail margin game of pharma business is bound to plateau after sometime and Entero beneficiary across it

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